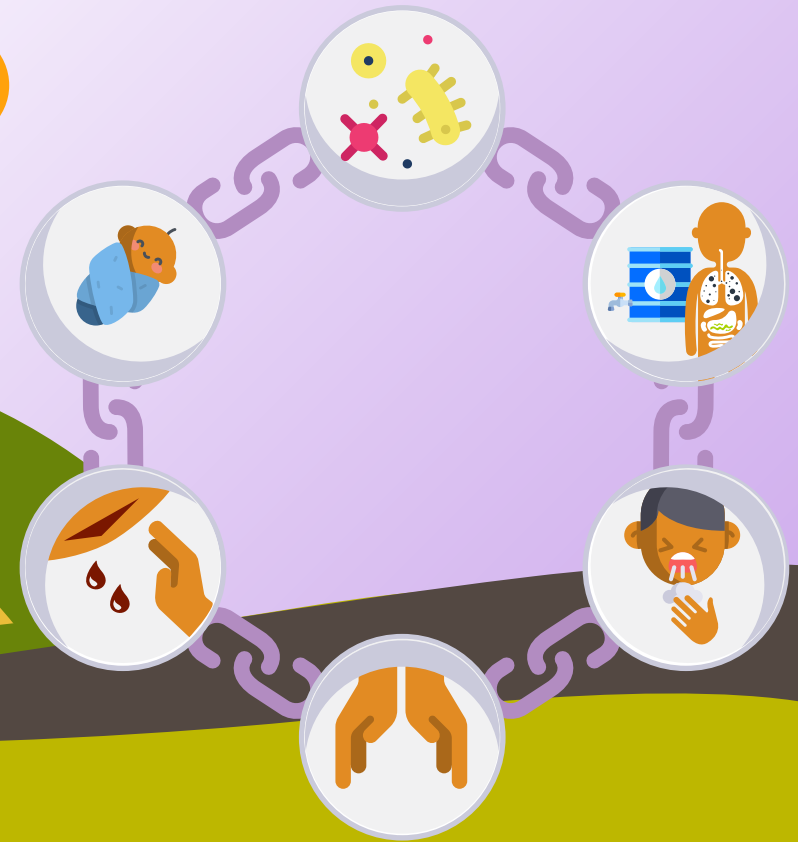
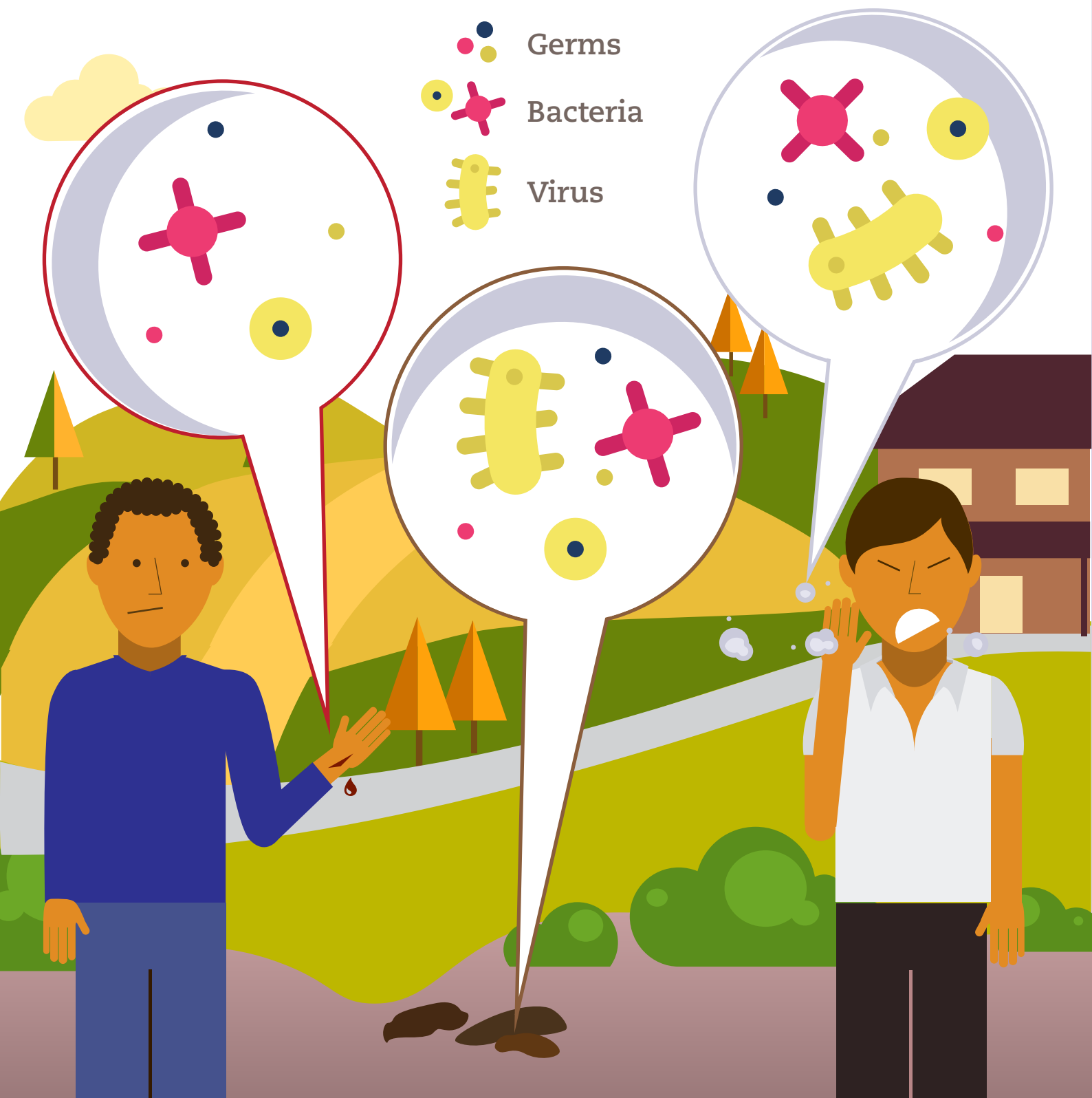
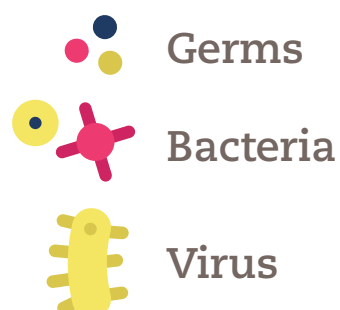




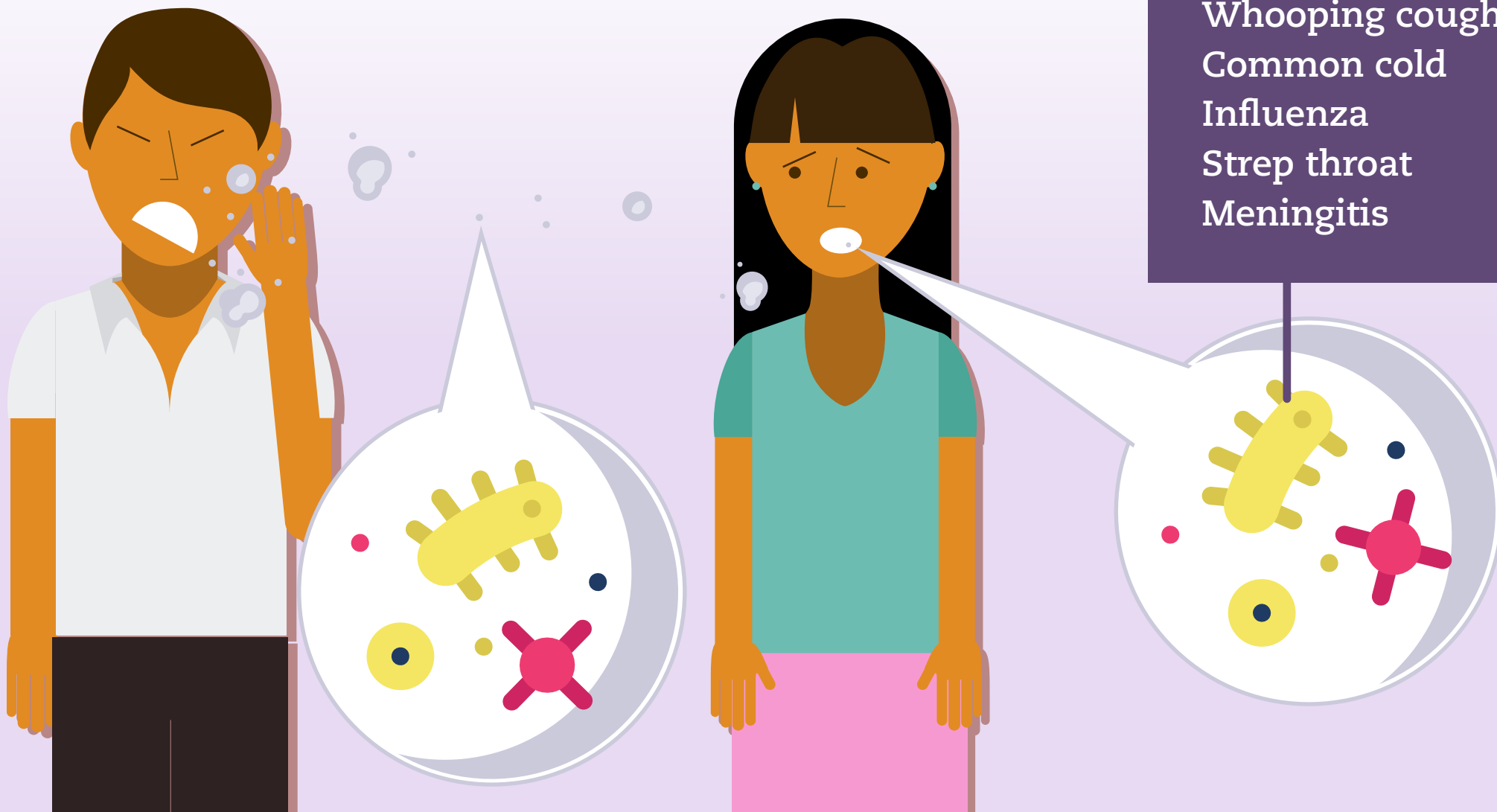


Basics of a communicable disease



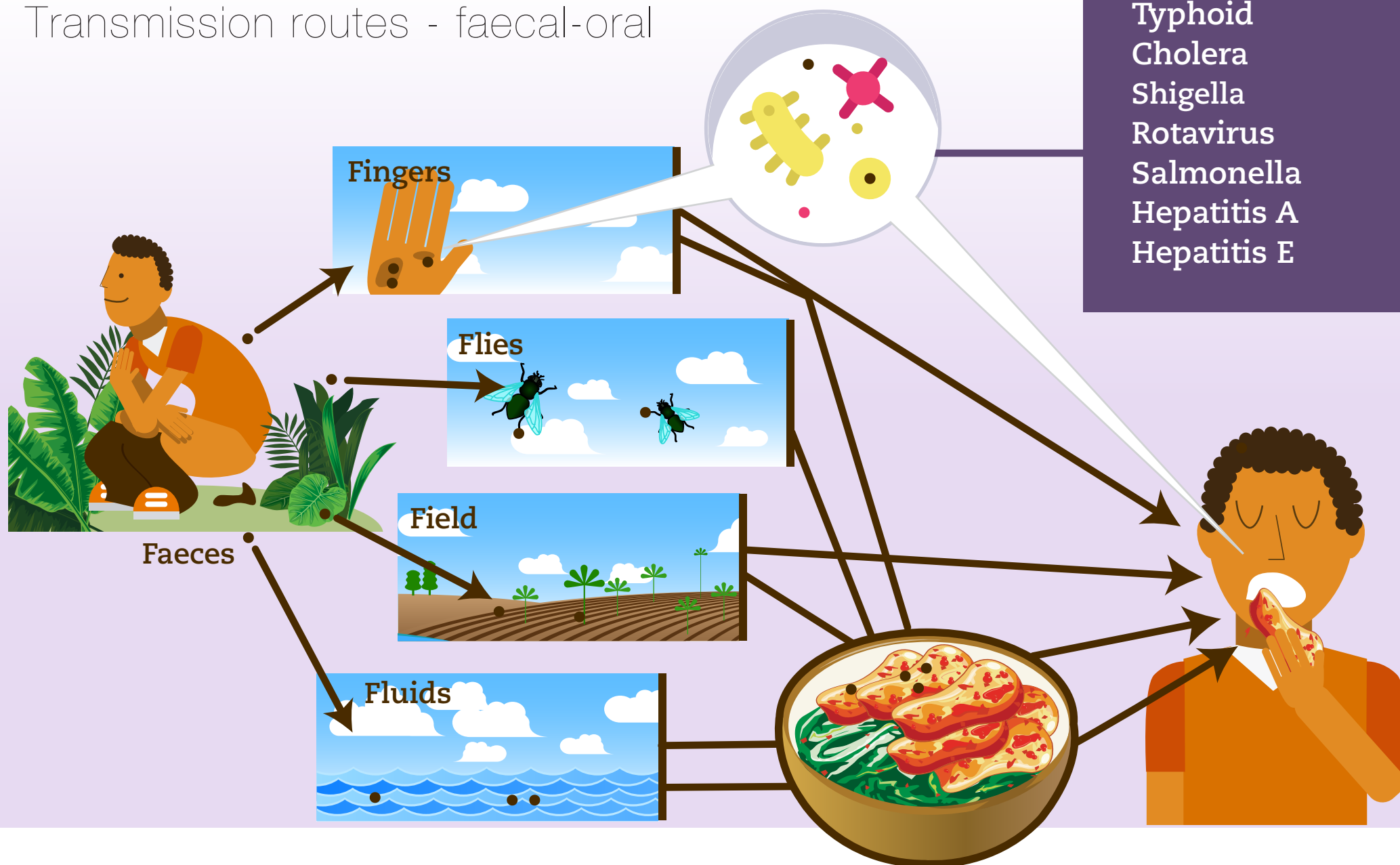


Transmission routes - air droplet



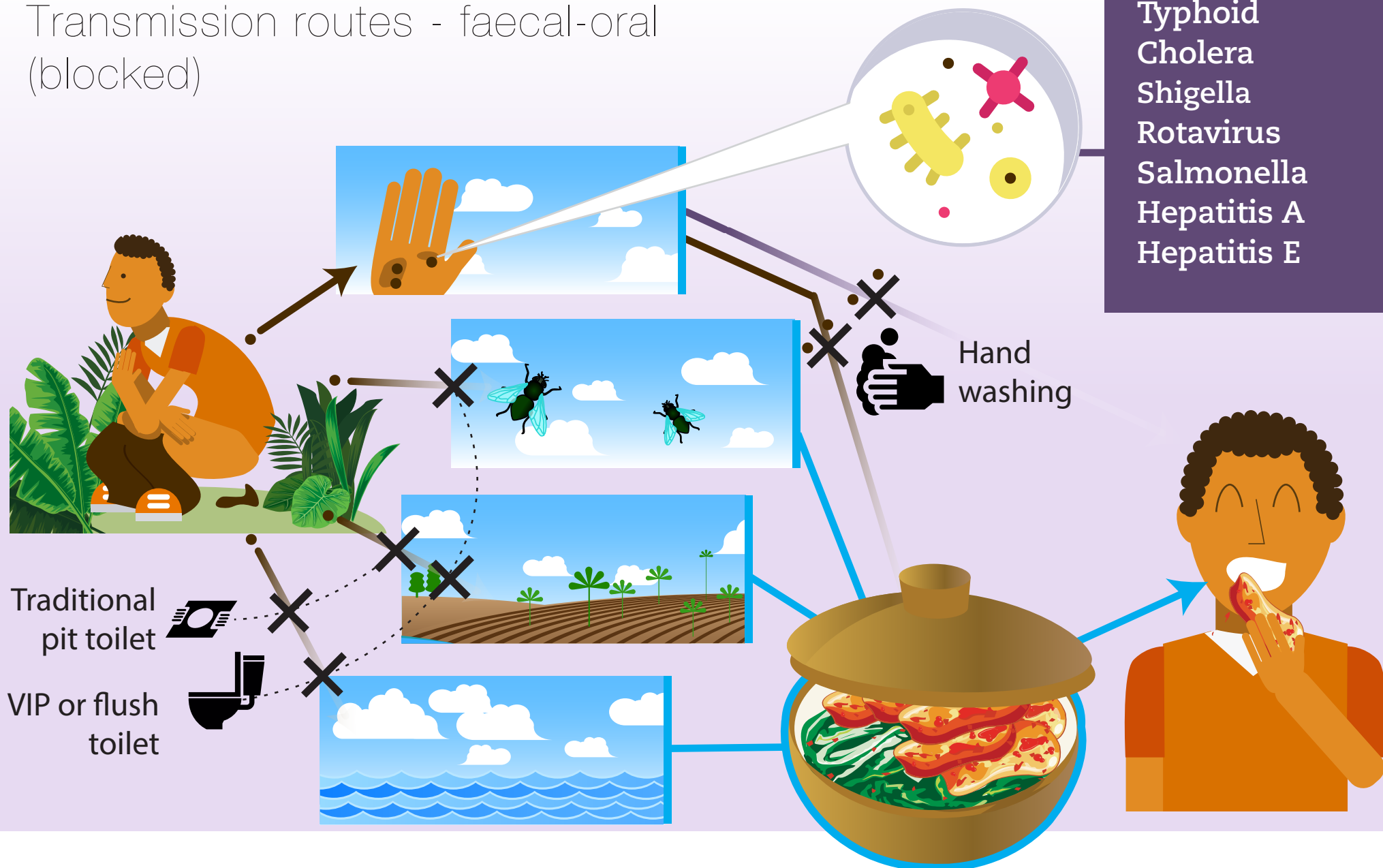


Transmission routes - faecal-oral



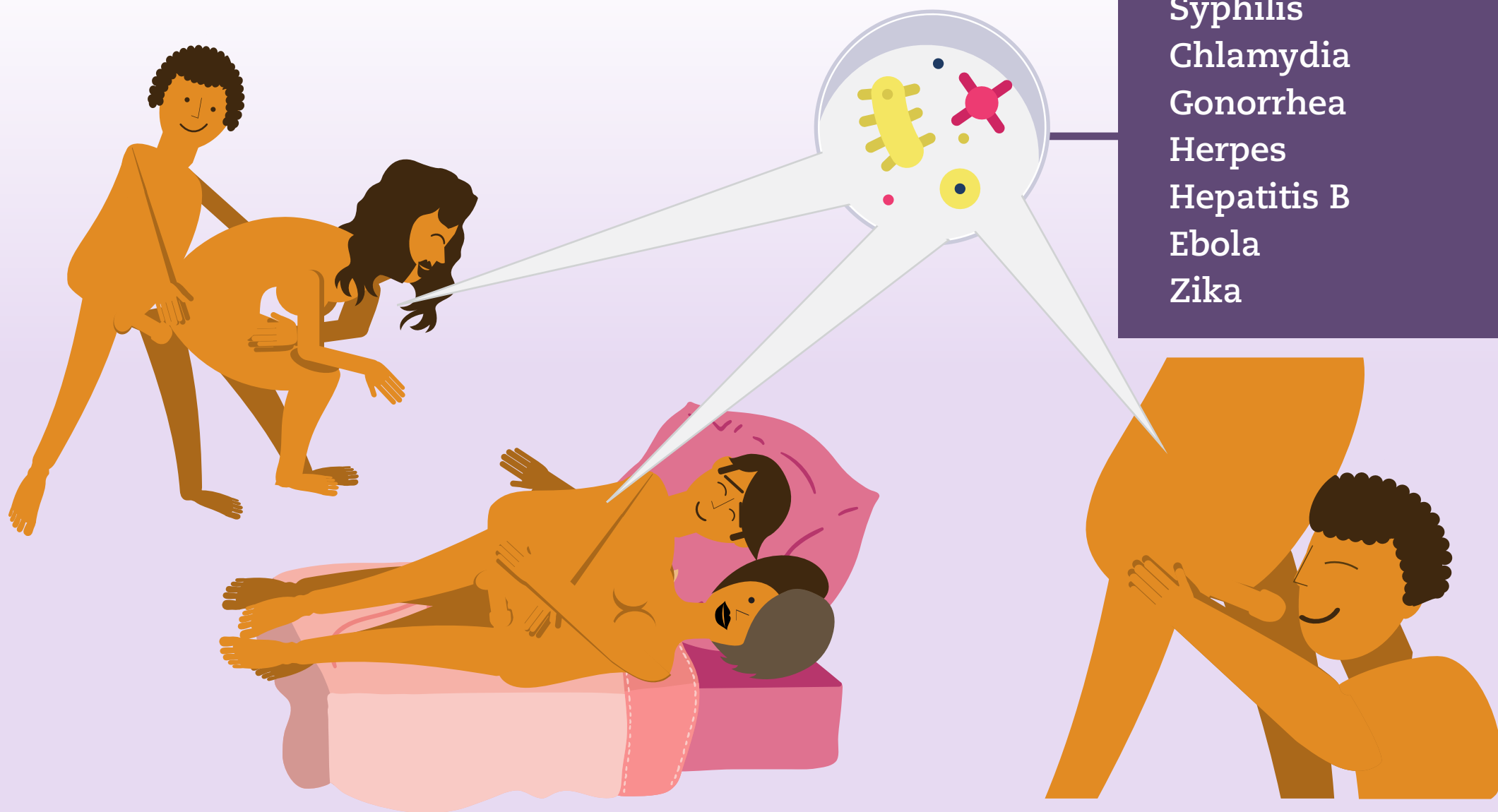


Acute water diarrhoea
Typhoid
Cholera
Shigella
Rotavirus
Salmonella
Hepatitis A
Hepatitis E



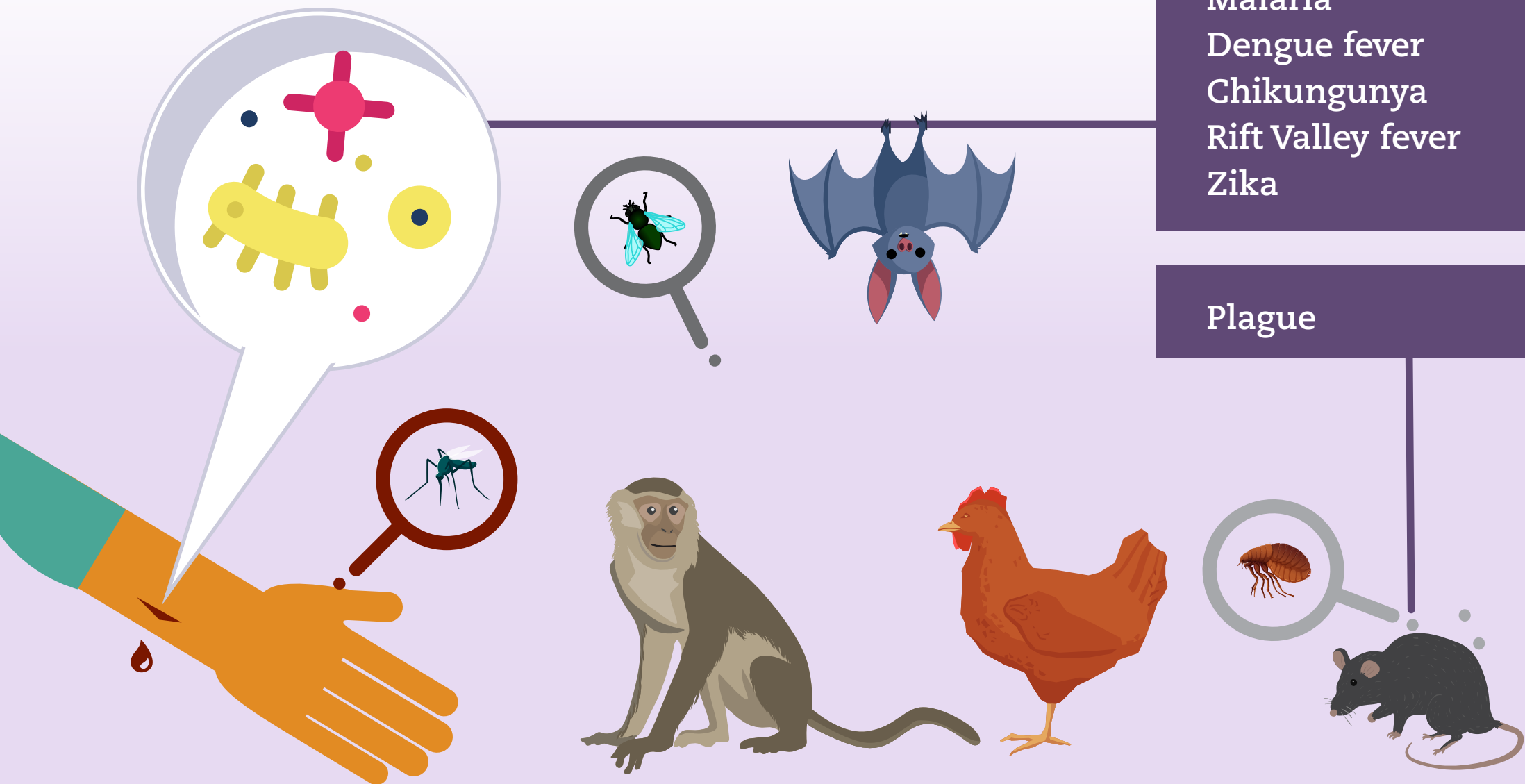


Transmission routes - sexual





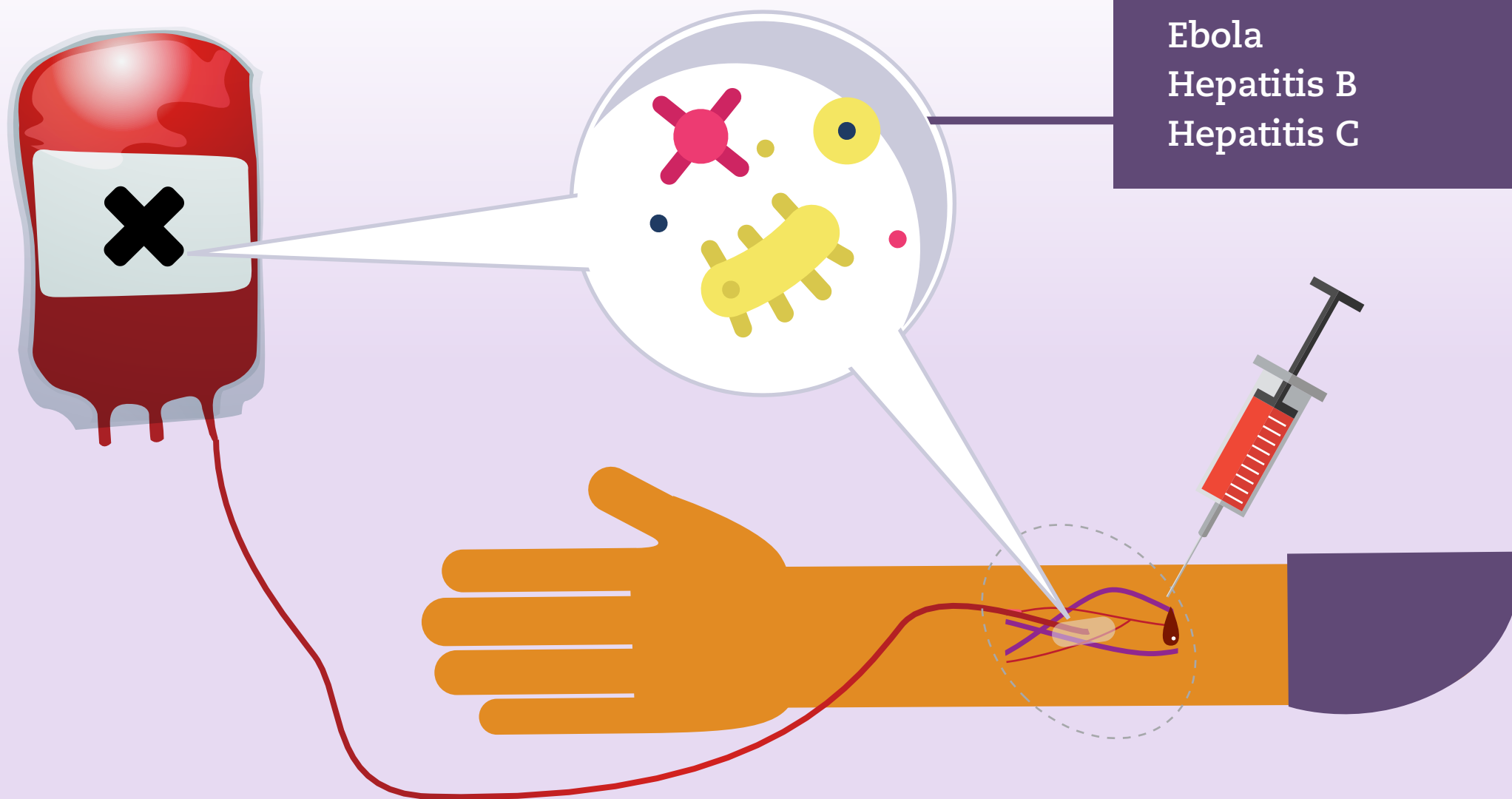
Transmission routes - vector-borne





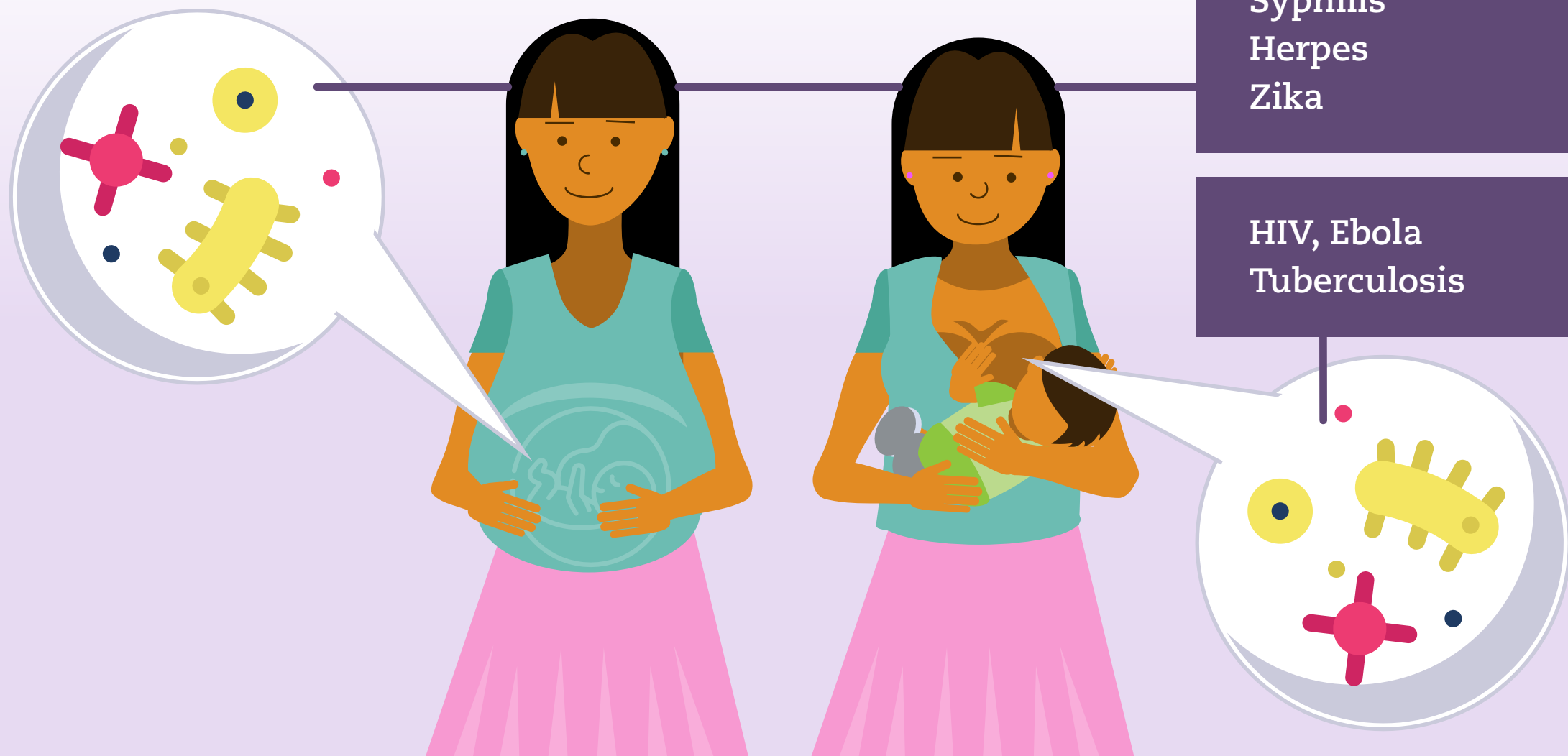
Transmission routes - blood

HIV
Ebola
Hepatitis B
Hepatitis C



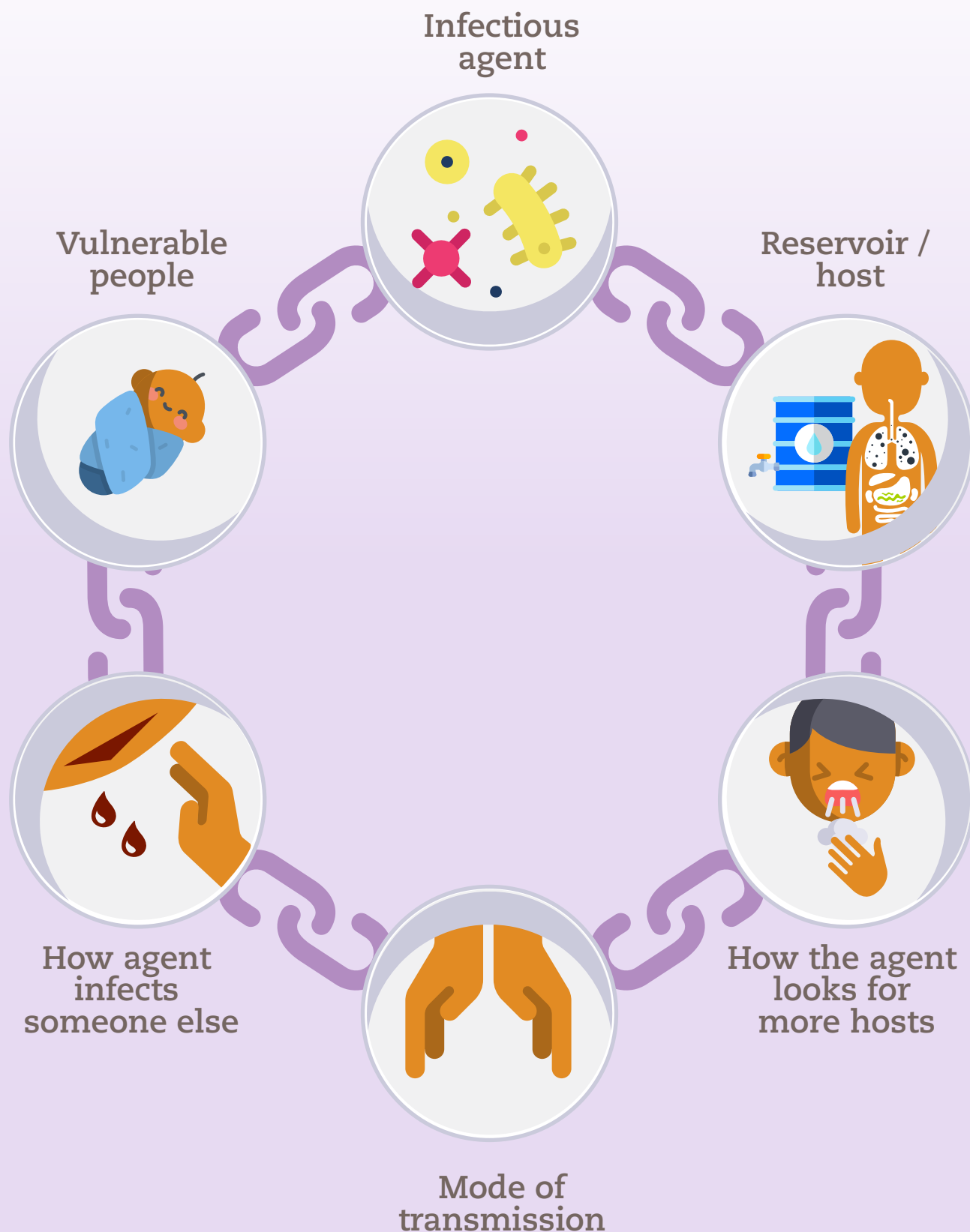


Transmission routes - mother to child





Chain of infection





Chikungunya

Transmission

Mosquito bite

Mosquitoes usually bite during the day, peaking during early morning and late afternoon/evening

Rarely from mother to newborn around the time of birth

Blood transfusion (possible)

Symptoms

Abrupt onset of fever frequently accompanied by joint pain

Muscle pain, headache, nausea, fatigue and rash

The joint pain is often very debilitating, but usually lasts only for a few days or may be prolonged to weeks

Most patients recover fully, but in some cases joint pain may persist for several months, or even years

In older people, the disease can contribute to the cause of death

Prevention

Protection against mosquito bites

Destruction of mosquito breeding sites around the house

Health promotion

Waste disposal and clean-up campaign

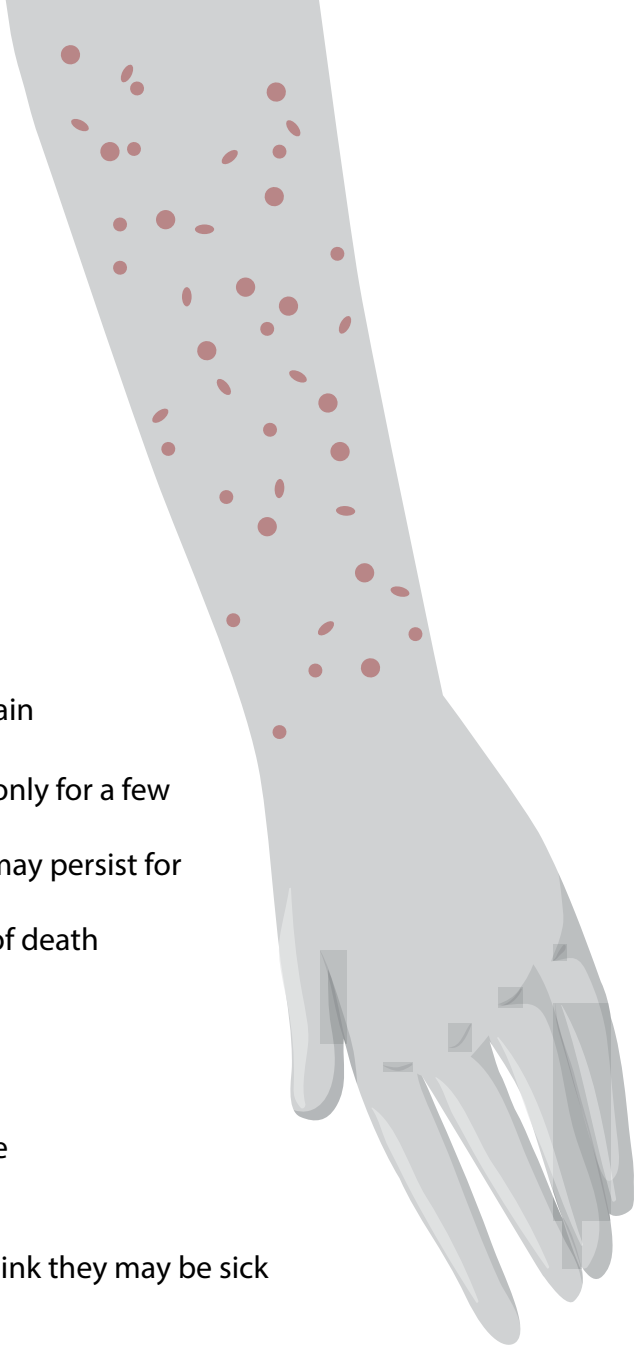
Knowledge of nearby clinic where to refer people who think they may be sick

Vulnerable people

Older people (over 65 years)

Newborns infected around the time of birth

People with medical conditions such as high blood pressure, diabetes, or heart disease



Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Who and where are the vulnerable people?

How many people are aware of Chikungunya?

Are there accessible health services and how far do you have to travel to get to them?

Is there a health education programme in place?

Are there any bed net distributions so that people can rest under a net during the day?

Is there a health promotion programme in place?





Chlamydia (bacteria)

Often sexually-transmitted infections (STIs) are missed because people have no symptoms or minor symptoms that disappear quickly but they can infect other people or cause problems when they become (or want to become) pregnant. The germs that cause STI's are carried in bodily fluids including semen, vaginal fluid and blood. The risk of transmission of diseases is always greater after a disaster due to overcrowding, a high stress environment and when there has been loss of lives. When our bodies are overstressed such as during or after a crisis or being in new surroundings, our bodies may be more vulnerable to infection. There can also be a risk of rape in displaced communities.

Transmission

Sexually transmitted infection (STI)

Unprotected vaginal, anal or oral sex with a person who has the infection

Can affect both men and women

Person may not show any symptoms but can pass onto other people through unprotected sex

Symptoms

Men and women may show no symptoms, or:

Redness at the opening of the penis

Stinging or burning when passing urine

Clear discharge from penis

Swelling of testicles if not treated

Vaginal discharge

Irregular bleeding (especially after sex)

Lower abdominal pain (can be during sex)

Stinging or burning when passing urine

Prevention

Sexually transmitted disease education

Promotion and availability of condoms if culturally appropriate

Promotion of safe sex

Provision of clean childbirth delivery kits for birthing mothers who have chlamydia

Handwashing and good hygiene practices

Health promotion

In case of a disaster there may be an increase of STIs

Promote safe sex practices and condom distribution

Promote reproductive health education

Promote good personal hygiene including handwashing

Advise anyone who thinks they may have an STI to go to a health facility for help

If appropriate encourage sexual partners to go to clinic for check up (contact tracing)

To keep safe from sexual assault which can pose risk of chlamydia infection:

- Is there lighting on way to latrines and water points?

- Is there security at night?





Chlamydia (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community? (do people have access to alcohol or other substances?)

Is this community a result of displaced people?

Who and where are the vulnerable people?

Are there orphans living in this community?

Who is responsible for their care?

Is there overcrowding?

Are there any health services available?

Are there any sexual and reproductive health programmes run in the community?





Common cold

Common cold is a high risk straight after an emergency when displaced people move into over-crowded living conditions. It is also a risk with ongoing extreme weather conditions.

Transmission

- Close contact with an infected person
- Air-borne germs spread through coughing and sneezing or direct contact with saliva of infected person with risk of virus being on hands
- Touching eyes, nose or mouth with unwashed hands
- Poor living conditions and poor ventilation
- Higher risk in emergencies when displaced people move into overcrowded living situations
- Can be any time of the year, but may be worse in cold or rainy weather



Symptoms

- | | | |
|------------|-------------|-------------|
| Runny nose | Sneezing | Sore throat |
| Cough | Watery eyes | Body aches |
| | | Fever |

Prevention

- | | |
|---|--|
| Avoid contact with infected person | Separation from people with cold/ acute respiratory infection |
| Hand hygiene | Exclusive breastfeeding up to and beyond six 6 months of age |
| Reduce overcrowding | Awareness of vulnerable people in community (under five-year-olds, elderly, people with poor immunity or malnutrition) |
| Use of good coughing and sneezing etiquette | |
| Improved ventilation in homes/shelters | |
| Health promotion | |

In case of an epidemic

- | | |
|--|---|
| Discuss isolation of sick people | Promote exclusive breast feeding up to and beyond six 6 months of age |
| Promotion of good ventilation inside homes | Awareness of vulnerable people in the community |
| Use of good coughing, sneezing etiquette | Health promotion |
| Hand hygiene | |
| Referral to health care | |

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

What are the risk habits in the community (e.g. overcrowding, extreme weather, smoking, poor ventilation inside homes, vulnerable people.

Is there access to safe water and soap for handwashing?
Is there a health education programme in place ?





Dengue fever

Dengue fever is a mosquito borne virus. The female Aedes mosquito becomes infected with dengue after biting sick people who have the virus in their blood. This is a daytime biting mosquito. Infected humans become the carriers. Severe dengue can be fatal.

Transmission

Most cases are from a mosquito bite that occurs early morning or early evening before dusk

In rare cases:

Dengue can be transmitted in organ transplants or blood transfusion from an infected person or from an infected mother to her foetus

Symptoms

High fever

Pain in the muscles and joints

Severe headache, often pain behind the eyes

Nausea and vomiting

In some cases the person can start to bleed

Skin rash two to five days after onset of fever

Haemorrhagic dengue:

Breathing distress

Severe abdominal pain

Ongoing vomiting and/or blood

Bruising

Bleeding gums and nose bleeds

Extreme fatigue

Prevention

Use insect repellent and mosquito coils to keep mosquitoes from biting you.

Use a bed net when resting during the early morning and late afternoon/evening.

Regularly remove standing water from around the home such as that which collects in buckets, pots, tyres and plastic rubbish to remove places where mosquitoes lay their eggs.

Regularly clean and scrub frequently used water containers or barrels. Scrub the edges of the water container at the water line to get rid of mosquito eggs.

Keep infrequently used water containers tightly covered with a lid that will not warp, dip or touch the water to prevent mosquitoes from getting in and laying their eggs.

Vulnerable people

People living in tropical areas

People with low immunity

Pregnant women

Volunteers' action

Motivate members in the community to destroy mosquito breeding sites.

Organize clean-up campaigns to eliminate standing water and mosquito breeding grounds.

Report if a community member shows symptoms and provide support.

Teach the Zika, Dengue and Chikungunya content from the ZDC toolkit if in Zika, dengue or chikungunya risk areas.

Provide information about how to keep Dengue-free.





Gonorrhoea (bacteria)

Often sexually-transmitted infections (STIs) are missed because people have no symptoms or minor symptoms that disappear quickly but they can infect other people or cause problems when they become (or want to become) pregnant. The germs that cause STI's are carried in bodily fluids including semen, vaginal fluid and blood. The risk of transmission of sexually-transmitted diseases is always greater after a disaster due to overcrowding, a high stress environment and when there has been loss of lives. When our bodies are overstressed such as during or after a crisis or being in new surroundings, our bodies may be more vulnerable to infection. There can also be a risk of rape in displaced communities.

Transmission

Sexually-transmitted infection

Unprotected vaginal, anal or oral sex with a person who has the infection

Can affect both men and women

Person may not show any symptoms but can pass onto other people through unprotected sex

Symptoms

Symptoms vary

Unusual vaginal discharge

Burning when peeing

Bleeding after sex

Abdominal pain

White or yellowish discharge from the penis

Burning when peeing

Itchy or stinging end of the penis

Swollen testicles (balls)

Prevention

Sexually transmitted diseases education

Promotion and availability of condoms if culturally appropriate

Promotion of safe sex

Provision of clean childbirth delivery kits if mother has gonorrhea to prevent spread to baby during birth

Handwashing and good hygiene practices

Health promotion

In case of a disaster there may be an increase of STIs

Promotion of safe sex practices and condom distribution

Promotion of reproductive health education

Promotion of good personal hygiene including handwashing

Advise anyone who thinks they may have an STI to go to a health facility for help

If appropriate encourage sexual partners to go to clinic for check up (contact tracing)

To keep safe from sexual assault which can pose risk of chlamydia infection:

- Is there lighting on way to latrines and water points?

- Is there security at night?





Gonorrhoea (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community? (do people have access to alcohol or other substances?)

Is this community a result of displaced people?

Who and where are the vulnerable people?

Are there orphans living in this community?

Who is responsible for their care?

Is there overcrowding?

Are there any health services available?

Are there any sexual and reproductive health programmes run in the community?





Head lice

Transmission

Anyone who comes in head-to-head contact with someone who already has head lice is at greatest risk. Spread by direct contact with the hair of an infested person

Less commonly, may spread by contact with clothing (such as hats, scarves, coats) or other personal items (such as combs, brushes, or towels) used by an infested person

Symptoms

Tickling feeling of something moving in the hair

Itching, caused by an allergic reaction to the bites of the head louse

Irritability and difficulty sleeping; head lice are most active in the dark

Sores on the head caused by scratching. These sores can sometimes become infected with bacteria found on the person's skin

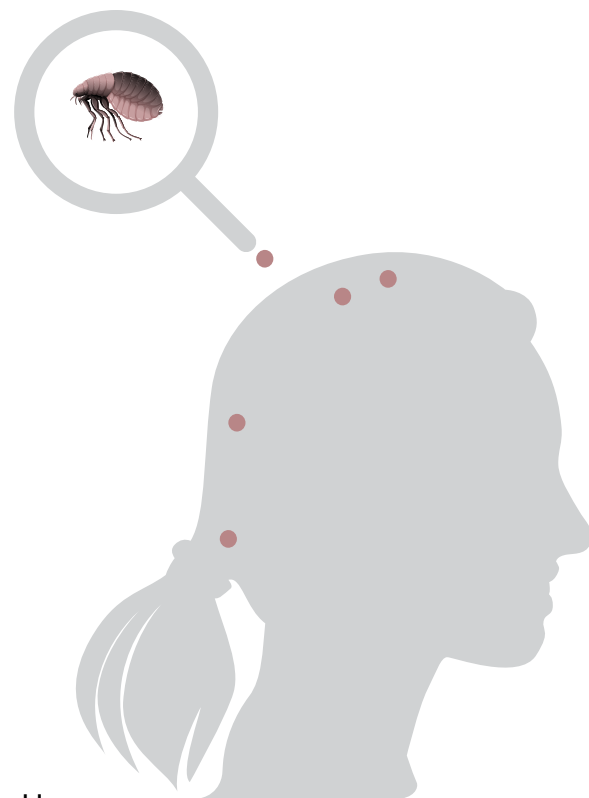
Prevention

Do not share items that touch the head like combs or towels

Avoid activities that lead to head-to-head contact

Keep belongings, especially upper body clothing, away from shared areas like coat closets

Keep long hair tied up





Hepatitis A

Hepatitis A is often a mild disease but can cause severe illness. Most people recover and are then immune. The risk is high when there is lack of access to safe water and sanitation and hygiene.

Transmission

Person-to-person contact (unwashed hands, object contaminated with human waste)

Food and water, mainly fruits, vegetables, shellfish and ice contaminated with human waste (faeces)



Symptoms

Some sufferers, especially children, show no signs of having the disease at all
Fatigue

Fever
Loss of appetite
Abdominal pain
Nausea

Dark urine
Yellowing of the skin or whites of the eyes (jaundice)

Prevention

Hand hygiene (handwashing with soap and safe water)
Clean water (good water sources, clean water container in household)
Good sanitation facilities (good latrines or defecation facilities)
Food prepared with good hygiene measures
Good shelter
Health promotion
Hepatitis A can be prevented by vaccine
Knowledge of nearby clinic where to refer people who think they may be sick

Vulnerable people

People living with an infected person
A sexual partner of an infected person
Someone who uses recreational drugs

People in areas where there is poor sanitation, poor access to safe water and inability to practise basic hygiene

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Who and where are the vulnerable people?
Where do people get their drinking water from? How is it collected? How is it stored? Is this area different from where people wash or go to the toilet?

What are the sanitation practices in the community?
What are the hygiene practices in your community?
Is there a vaccination programme in place?
Is there a health promotion programme in place?





Hepatitis B (virus)

The Hepatitis B virus lives in a person's liver and uses the bloodstream as a way to get around the body. A healthy liver cleans the blood, helps digest food, protects us from infection and helps stop bleeding so hepatitis B is a serious disease that needs monitoring and treatment. Hepatitis B can live inside for a long time without any symptoms or sickness. It is important for people with Hepatitis B to get treatment.

Transmission

Close contact with blood, semen or other bodily fluids from an infected person
Open wounds (cuts), sexual contact
Mother to baby (foetus or during birth)
Needle contact (sharing needles)



Symptoms

Fatigue	Abdominal pain
Nausea	Dark urine
Vomiting	Yellowing of the skin or whites of the eyes (jaundice)

Some people go on to develop chronic infection including long-term sickness of their liver

Prevention

Hepatitis B vaccination for newborn
Awareness of country vaccination programme for older children and specific workers
Promotion of safe sex practices and condom distribution
Teaching about awareness of protecting and cleaning open wounds
Hand hygiene and health promotion
Transmission of disease education
Knowledge of nearby clinic where to refer people who think they may be sick
Screening for hepatitis B and C in blood products

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

What are the risk habits in the community (intravenous drug use (injecting), sexual practices)?
What are the hygiene practices in this community?
Who and where are the vulnerable people?
What are the burial procedures and practices in the community?
What are the living conditions of people in the community?
Are there any sexual and reproductive health programmes run in the community?





Hepatitis C (virus)

The Hepatitis C virus lives in a person's liver and uses the bloodstream as a way to get around the body. A healthy liver cleans the blood, helps digest food, protects us from infection and helps stop bleeding. Hepatitis C is a serious disease that needs monitoring and treatment and is very contagious. Hepatitis C sickness can last a lifetime and it is important to get treatment.

Transmission

- Blood-borne illness
- Transfusion of unscreened blood products
- Needle contact (sharing needles) when injecting for drug use
- Sexually transmitted
- Can be passed from mother to baby (rare)
- Can be an acute or chronic illness that lasts a lifetime



Symptoms

- | | |
|------------------------------|---|
| Fatigue, fever and decreased | Dark urine |
| Decreased appetite | Joint pain |
| Nausea and vomiting | Jaundice (yellow skin and whites of eyes) |
| Abdominal pain | |

Prevention

- Hand hygiene
- Safe handling and disposal of needles and sharps
- Promotion of safe sex practices and condom distribution
- Screening for hepatitis B and C in blood products
- Health promotion
- Education programme for people with hepatitis C
- Knowledge of nearby clinic where to refer people who think they may be sick

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

- How many people live in the affected community?
- What are the risk habits in the community?
- Who and where are the vulnerable people?
- Are recreational drugs used in this community and if so does the habit involve using needles to inject?

- Is there an area people go to use drugs in this community?
- Are there accessible health services and how far do you have to travel to get to them?
- Do they give blood transfusions at the facility?





Hepatitis E

Hepatitis E is a liver infection that most commonly enters the body via the gut (intestine) through drinking contaminated water. The disease is normally self limiting after six weeks. There, however, is a risk of serious illness developing for some people. There is a vaccine in development.

Transmission

Faecal oral route due to contaminated water
Person-to-person contact (unwashed hands, object contaminated with human waste)
Ingestion of uncooked/undercooked pork or deer meat, meat products or organs (e.g. liver) and shellfish
Blood transfusion of unscreened blood
During pregnancy to foetus



Symptoms

Sufferers often show no signs of having the disease at all

Mild fever	Nausea	Yellowing of the skin or whites of the eyes (jaundice)
Loss of appetite	Dark urine	May have skin rash or itchy skin
Abdominal pain	Pale stools	In rare cases hepatitis E can be severe and these patients (mainly pregnant women) are at risk of death

Prevention

Clean water (good water sources, clean water container in household)	safe water)
Good sanitation practices and facilities (good latrines or defecation facilities)	Food prepared with good hygiene practices
Proper hygiene (handwashing with soap and	Health promotion
	Knowledge of nearby clinic where to refer people who think they may be sick

Vulnerable people

Pregnant women, particularly those in second and third trimester
Persons with pre-existing chronic liver disease

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Who and where are the vulnerable people?
How many pregnant women live in the affected community
Where do people get their drinking water from? How is collected? How is it stored? Is this area different from where people wash

or go to the toilet?
What are the sanitation practices in this community?
Is there a health promotion programme in place?





Herpes simplex virus (HSV)

Herpes is a sexually-transmitted disease (STI). Even though most people do not have symptoms they can still spread the disease to other sexual partners. The germs that cause STI's are spread through vaginal, anal or oral sex. Herpes can be passed to a newborn baby most commonly during delivery. The risk of transmission of sexually-transmitted diseases is always greater after a disaster due to overcrowding, a high stress environment and when there has been loss of lives. As to Viv's request for more info, add - When our bodies are overstressed such as during or after a crisis or being in new surroundings, our bodies may be more vulnerable to infection. There can also be a risk of rape in displaced communities.

Transmission

There are two types (HSV1 and HSV2)

HSV 1

Oral to oral contact

Cold sores (ulcers or blisters on lips)

Can also cause genital herpes through oral sex (ulcers on penis or outside of vagina)

HSV 2

Sexually-transmitted infection

Unprotected vaginal, anal or oral sex with a person who has the infection

Mild symptoms

Small ulcers (like a pimple) on the genital area or blisters (genital herpes)

Ulcers heal but come back



Symptoms

Initially, small shallow ulcers heal but often return if untreated

May have fever, body aches or swollen glands

Comes back in cycles

Prevention

Sexually-transmitted diseases education including monogamous relationship (one partner)

Promotion of safe sex and availability of condoms if culturally appropriate

Avoid sex when partner has symptoms

Handwashing and good hygiene practices

Health promotion

Provision of clean childbirth delivery kits

In case of an epidemic

Promotion of safe sex practices and condom distribution

Promotion of reproductive health education (young people who want to have a baby)

Promotion of good personal hygiene including handwashing

Advise anyone who thinks they may have an STI to go to a health facility for help

If appropriate encourage sexual partners to go to clinic for check up (contact tracing)

To keep safe from sexual assault which can pose risk of chlamydia infection:

- Is there lighting on way to latrines and water points?

- Is there security at night?





Herpes (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community? (do people have access to alcohol or other substances?)

Is this community a result of displaced people?

Who and where are the vulnerable people?

Is there overcrowding?

Are there any health services available?

Are there any sexual and reproductive health programmes run in the community?





HIV

HIV is a major public health issue across the world. According to WHO (2017) there are over 36.9 million people living with HIV, the African region being the most affected with in excess of 25.7 million people living with HIV.

HIV is a virus (human immunodeficiency virus) that weakens a person's immune system making it difficult to fight against infections, cancers and other diseases. The virus works by destroying the cells responsible for fighting infections (immune cells).

Once HIV progresses to the most advanced stage it is known as AIDS (Acquired immunodeficiency syndrome). This can take many years to develop and is determined by the identification of certain infections, symptoms and certain cancers. It is important to know that although there is no cure for HIV it can be controlled with proper medical care.

Transmission

Exchange of bodily fluids from an infected person such as semen, vaginal fluids, blood and breast milk (less common)

Must come in contact with broken mucous membrane, tissue or open bleeding wounds

or injected directly into bloodstream (via contaminated needle)

Cannot be transmitted by kissing, hugging, sharing food, water or shaking hands

Symptoms (variable)

Flu like symptoms such as:

Fever

Headache

Sore throat

Rash

Weight loss

Swollen lymph nodes

Diarrhoea

Chronic cough

Mouth ulcers

Oral thrush

Itchy rash

Prevention

Promotion of safe sex and availability of condoms if culturally appropriate

Disease transmission awareness including sexually-transmitted diseases

Provision of clean delivery kits

Health promotion

Use of personal protection equipment (PPE) if caring for a HIV person who is acutely unwell

Promotion of safe injecting practices (if contextually appropriate)

In case of a disaster there may be an increased risk of HIV

Awareness of HIV people within the community who may need added support

Promotion of safe sex practices and condom distribution

Promotion of good personal hygiene including handwashing

Health promotion

Referral of known cases to health facilities for ongoing care

Awareness of vulnerable people in the community

There is an increased risk of transmission of HIV after an emergency due to overcrowding and potential increase of sexually-transmitted diseases.





HIV (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community e.g. contact with people who have:

- People living with TB
- Unsafe sexual practices and STIs
- People living with HIV
- Chronic medical conditions especially respiratory illness
- Malnutrition

What HIV programmes are available and are they accessible to people in this community?

Are there HIV health protocols in place i.e. where do people go for antiretroviral treatment?

Who and where are the vulnerable people?

Are there any accessible health services?

Are there any sexual and reproductive health programmes run in the community?





Leptospirosis

Leptospirosis is an infection caused by bacteria spread by the urine of rodents or other animals. It is potentially fatal if untreated. The infection enters the body through breaks in the skin or mucosa.

Transmission

Direct exposure into open wounds or mucosa (eyes, nose or mouth) via infected urine or soil infected with urine

Ingestion of water contaminated with urine from infected animals

Person to person transmission is rare

Symptoms

Fever

Severe headache

Chills

Muscle pain

Vomiting

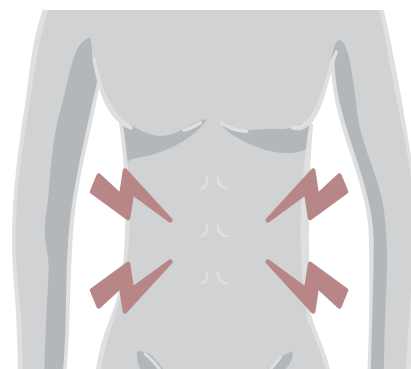
Diarrhoea

Abdominal pain

Jaundice

Skin rash

Red eyes



Prevention

Avoid contact with animal excreta

Avoid walking through flooded water

Wear protective footwear when walking through soil or water that may be contaminated

Good hygiene practices if working with or handling animals

Vulnerable people

Farmers

Agricultural workers

Sewerage workers

People who handle waste and rubbish

Anyone who is exposed to flood waters

In case of an epidemic

Encourage people not to swim or walk in flooded areas

Promotion of wearing footwear in muddy areas, waste areas and flood waters

Promotion of protective clothing for high risk workers

Promotion of dedicated areas (waste pits) for rubbish and solid waste

Refer anyone who has symptoms to health care facilities

Promotion of safe clean water collection and storage

Hand hygiene





Leptospirosis (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Record details such as rubbish areas, solid waste areas, drainage areas, water points and latrines.

Do parts of the community flood?

When did people start to fall sick? How many?

Is this a community of displaced people?

Are there high risk workers in this community i.e farmers, rubbish collectors, agricultural workers, sewerage workers, rescue workers if in a flood?

Where do people obtain their drinking water? Is the source contaminated with flood water? Do people treat their water?

What sanitation facilities (including communal latrines) are available? Do people use them?

What handwashing facilities are available (at households, in markets, etc.)? Do they have soap?

Where are the local health facilities and services? (Include traditional and community carers.)

What are the community's habits, practices and beliefs about hygiene, sanitation and water?

Are there health promotion activities already in place?





Malaria

A mosquito-borne disease. The female Anopheles mosquito infected with the malaria parasite transmits the disease via a bite. It is preventable and treatable, but can be fatal if untreated. A vaccine has been developed against the most deadly type, P.Falciparum and is in pilot phase.

In endemic countries people may have partial immunity and not show any symptoms.

Transmission

Mosquito bite at night (between dusk and dawn)

Symptoms

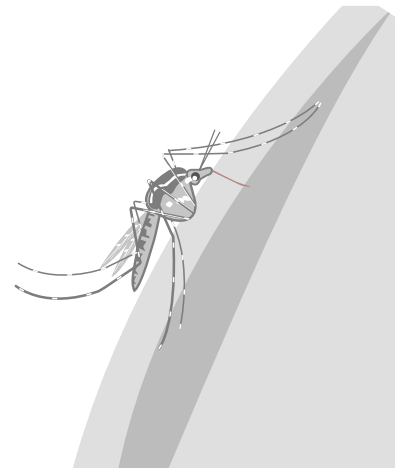
Starts with several days of fever, headaches, and chills

May be mild or severe

In very severe cases, weakness, loss of consciousness, severe anaemia, acute respiratory and renal failure

Children may develop severe anaemia which is very dangerous

Respiratory distress



Prevention

Prevention of mosquito bites

Use of mosquito nets over beds and screens on windows

Spraying of houses to kill mosquitoes

Use of antimalarial drugs

Elimination of mosquito breeding sites

Health promotion

Use of anti-malarials

Knowledge of nearby clinic where to refer people who think they may be sick

Solid waste management

Vulnerable people

Children under five years

Pregnant women

People living with HIV

Mobile populations and travellers

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the community?

How many children under five years of age live in the community?

Do people usually use insecticide-treated mosquito bednets?

Who and where are the vulnerable people?

Are there accessible health services and how far do you have to travel to get to them?

Do people take antimalarial drugs?

Is there a health promotion programme in place?

Is anyone distributing mosquito nets?

What are the sanitation practices?

What is the community doing about mosquito breeding sites?





Measles

Measles is a highly contagious infectious disease that is a high risk after every emergency due to population movement, overcrowding and when there are low levels of vaccination coverage. There is a vaccine for measles and it is common after an emergency for a mass measles vaccination campaign to be rolled out to minimize chances of outbreak. Measles immunity needs a high vaccination coverage (i.e WHO recommends 95 per cent coverage to create herd immunity).

Transmission

Droplets of saliva/mucous from one person to another, mainly by coughing and/or sneezing
Close personal contact or direct contact with infected nasal or throat secretions

Symptoms

Usually starts with high fever lasting for a few days
Runny nose
Rash starting on face and upper neck and behind head and ears which can last up to 14 days
Complications are most common in children under five years or those with weakened immune systems e.g. persons living with HIV



Prevention

Routine vaccination of children (by injection)
Mass immunization campaigns in countries with high case and death rates
Rapid detection and referral of suspected cases to health facilities
Infected people should be isolated for four days after they develop a rash
Health promotion
Coughing etiquette

Knowledge of nearby clinic where to refer people who think they may be sick
Get to know health workers who are doing vaccinations and the centres where they are done
Know children and families who are hard to reach or unvaccinated for other reasons, and help them reach vaccination centres
Help health workers make sure that all children are vaccinated

Vulnerable people

Children who are not vaccinated, especially those poorly nourished and with vitamin A deficiency
Any unvaccinated adult

People with compromised immune systems, such as from leukaemia and HIV infection
Unvaccinated pregnant women
Displaced populations





Measles (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the community?

How many children under five years of age live in the community?

How many children have not received any or all of their measles vaccinations?

Who and where are the vulnerable people?

Are there any health services and where?

Are there any vaccination campaigns planned?

Is there a health promotion programme in place?





Pandemic influenza

Pandemic is when there is a world-wide emergence of a strain of influenza A, which can spread rapidly from person to person.

Transmission

Like respiratory infections, air-borne droplets of saliva/mucous from one person to another, mainly while coughing or sneezing

Symptoms

Is a severe kind of influenza and therefore can have very similar symptoms to regular influenza, such as sudden illness, fever, cough and shortness of breath

May be accompanied by chills, runny nose, sore throat, tiredness and feeling unwell or loss of appetite

All of these symptoms may be very severe if the germ that develops is very strong



Prevention

Social distancing
Hand hygiene
Respiratory etiquette
Household ventilation

Health promotion
Coughing etiquette
Knowledge of nearby clinic where to refer people who think they may be sick

Vulnerable people

Elderly people over 65 years of age

Weakened immune system

In case of an epidemic

Separation of patients with respiratory symptoms from those presenting with other symptoms
Social distancing
Community mobilization
Health promotion

Detection and referral of new and suspected cases to health facilities
Coughing etiquette
Hand hygiene
Knowledge of nearby clinic where to refer people who think they may be sick

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Who and where are the vulnerable people?

Has the population received the most recent influenza vaccination?

Is there a health promotion programme in place?





Plague

Plague is an infectious disease found in fleas carried on mammals. People contract the disease if they are bitten by an infected flea. This type of the disease is called bubonic plague and it is the most common form of the disease. If the bacterial infection gets inside the lungs the disease is known as pneumonic plague. It can be fatal and is more severe than bubonic. Both of these diseases are treatable and preventable.

Transmission

A bite from an infected flea

Person to person via inhalation of infected air droplets (pneumonic)

Touching contaminated animal parts or infectious bodily fluids

Symptoms

Fever

Headache

Muscle pain and weakness

Chills

Nausea and vomiting

Tender lymph nodes (bubo) - hard lumps that can turn into sores filled with pus

Pneumonic – shortness of breath, chest pain, productive cough

Prevention

Decrease rodents around households

Awareness of fleas and protection from same

Safe handling of animal carcasses

Promotion of basic hygiene practices including handwashing

Promotion of good food storage in containers with lids

Social mobilization and behaviour change

Vulnerable people

Everyone in the community

In case of an epidemic

Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)

Awareness of safe disposal of animal carcasses

Environmental control of fleas

Awareness of safe handling of sick animals

Increase social mobilization and health promotion activities

Refer anyone who has symptoms to health care facilities

Isolation of people who are sick with the plague

Wearing of personal protective equipment for caregivers





Plague (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Has anyone fallen sick with plague symptoms?

How many and where?

Has anyone died?

How do people store their foods?

Do people have unscreened homes?

Do people sleep on the ground or on raised beds?

Where are the local health facilities and services? (Include traditional and community carers.)

What are the community's habits, practices and beliefs about slaughtering and handling of animals?

Are there health promotion activities already in place?

Are there rumours about the plague spreading?





Polio

Polio is a highly contagious disease with young children under five years of age being the most at risk. The disease is linked to poor personal hygiene. Outbreaks are at a risk where immunity levels are low (less than 80 per cent coverage), and where there are overcrowded living conditions and poor sanitation. It can be fatal if it affects the respiratory muscles.

Transmission

Faecal oral route - drinking contaminated water or eating contaminated food

Person to person contact – touching contaminated items and then putting hands in mouth

Symptoms

Mostly just mild symptoms

Starts with:

Fever

Tiredness

Headache

Can progress to stiffness in back, neck and limbs

Muscle aches or weakness in legs or arms

Paralysis occurs in some cases (one in 200) – usually only on one side

Prevention

Polio vaccine (oral OPV) - multiple doses depending on country protocol and type of vaccine (lifetime protection)

Promotion of basic hygiene practices including handwashing

Promotion of safe sanitation practices

Social mobilization and behaviour change

Vulnerable people

Children under five years of age

Children who are unvaccinated against polio

Children living in overcrowded conditions with poor access to safe water, sanitation and basic hygiene

In case of an epidemic

Promotion of mass vaccination campaigns

Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)

Increase social mobilization and health promotion activities

Refer anyone who has symptoms to health care facilities

Promotion of safe clean water, collection and storage

Promotion of safe sanitation practices

Hand hygiene





Polio (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Has anyone fallen sick with polio symptoms?
e.g. paralysis (acute flaccid paralysis (AFP))?

Are any children sick with polio symptoms?
Where are they?

Are there unvaccinated children in this community?

Is there a vaccination campaign planned for this community?

What sanitation facilities (including communal latrines) are available? Do people use them?

What handwashing facilities are available (at households, in markets, etc.)? Do they have soap?

Where are the local health facilities and services? (Include traditional and community carers.)

What are the community's habits, practices and beliefs about hygiene, sanitation and water?

Are there health promotion activities already in place?





Respiratory infections

Transmission

Droplets of saliva/mucous from one person to another, mainly while coughing or sneezing

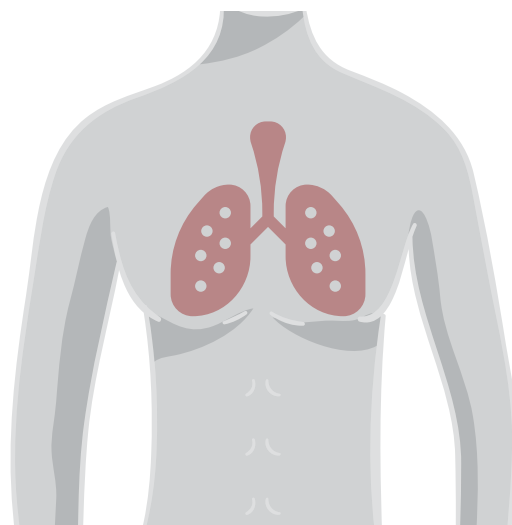
Symptoms

Wide range of symptoms
Look for fever, cough and difficulty breathing

Prevention

Identification of patients before they spread the disease to other people
Health promotion
Improve ventilation in shelters
Reduce overcrowding in shelters

Improve the nutritional situation, especially of children
Coughing etiquette
Knowledge of nearby clinic where to refer people who think they may be sick



Vulnerable people

Everyone in the community can get respiratory infections
People are more at risk in overcrowded and displacement situations
People with low immunity
Children under five years

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many children under five years of age live in the community?
What is the shelter situation in the community? How many people live in each household?
Who and where are the vulnerable people?
Are there any health services and where?
Is there a health promotion programme in place?
What are the hygiene practices?





Rift Valley fever

Rift Valley fever is a virus that affects animals and also humans. Most people are infected via direct contact with blood or animal tissue when handling them. The most likely animals are livestock and the most likely contact is during the butchering process.

Transmission

- Direct contact with infected blood or organs of infected animals via a break in skin or inhalation
- Mosquito bite from infected mosquitoes
- Drinking unpasteurised milk of an infected animal

Symptoms

- Fever
- Headache
- Muscle and joint pain
- Some people develop neck stiffness

- Sensitivity to light in the eyes, can progress to decreased vision
- Decreased appetite
- Sometimes vomiting
- Sometimes jaundice or bleeding from nose or mouth (most severe)

Prevention

- Promotion of animal vaccination
- Promotion of basic hygiene practices including handwashing

- Promotion of good food hygiene especially thorough cooking of meat
- Social mobilization and behaviour change
- Vector control

Vulnerable people

- People working with livestock who are in contact with blood or organs

In case of an epidemic

- Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)
- Awareness of safe handling and slaughtering of animals
- Awareness of safe disposal of animal carcasses
- Awareness of safe handling of sick animals
- Increase social mobilization and health promotion activities
- Refer anyone who has symptoms to health care facilities
- Promotion of good food hygiene practices including thorough cooking of meat and safe preparation of milk
- Hand hygiene
- Vector control





Rift Valley fever (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Has anyone fallen sick with Rift Valley fever symptoms? How many and where?

Has anyone died?

Do people cook meat thoroughly?

Do people boil milk?

What handwashing facilities are available in the community, at the marketplace and where livestock are handled? Do they have soap?

Where are the local health facilities and services? (Include traditional and community carers.)

What are the community's habits, practices and beliefs about slaughtering and handling of animals?

Do you know of any markets selling infected animal meat? Where?

Are there health promotion activities already in place?





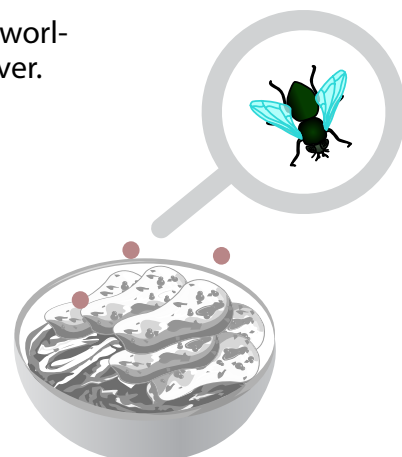
Salmonella (bacteria)

Salmonella is caused by a bacteria. It is a major cause of food-borne sickness worldwide and most commonly caused by contaminated food. Most people recover.

Transmission

Food-borne illness
Ingestion of contaminated food (meat, eggs, chicken, milk)
Person to person via faecal oral route

Poor food preparation
Poor living conditions with poor hygiene, inadequate sanitation or safe water
Serious risk in disasters



Symptoms

Diarrhoea, fever, abdominal pain
Nausea or sometimes vomiting, headache

Mostly relatively mild but need to watch for severe cases with dehydration

Prevention

Control of food processes from agriculture to safe food preparation and storage of food
Thorough cooking of food
Good hygiene practices
Avoid drinking raw milk

Boil water if source not safe
Health promotion
Safe sanitation practices
Washing hands after changing babies, nappies

In case of an epidemic

Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)
Detect cases and refer severe to health care facilities
Promotion of safe food preparation and storage

Hand hygiene and safe sanitation practices
Promotion of safe clean water
Isolation of sick people
Awareness of vulnerable people in the community (very young, elderly and malnourished)
Health promotion

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

When did people start to get sick with diarrhoea?
How many have fallen sick in the community?
Has anyone died from diarrhoea in this community?

What are the hygiene practices in this community?
What are the food preparation practices?
Who and where are the vulnerable people?
Where do people get their meat from and how is it prepared?
What do people think has caused this sickness?





Shigella (bloody diarrhoea)

Shigella is also known as bloody diarrhoea. It is an infection caused by bacteria. There is no vaccine for shigella so it is a high risk for an epidemic to occur with a potential for high death rate if not treated.

Transmission

- Faecal oral route
- Person to person contact
- Ingestion of poorly prepared food or unsafe water
- Poor living conditions with poor hygiene, inadequate sanitation or safe water
- Serious risk in disasters

Symptoms

- Bloody diarrhoea
- Abdominal pain and/or cramping
- Fever



Prevention

- Access to safe clean drinking water
- Safe sanitation practices
- Handwashing and good hygiene practices
- Safe preparation and storage of food

- Refer cases to health care facilities for treatment
- Health promotion
- Social mobilization and behaviour change

In case of an epidemic

- Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)
- Detect cases and refer to health care facilities
- Promotion of safe clean water
- Promotion of safe sanitation options including disposal of faeces from babies and small children
- Hand hygiene
- Isolation of sick people
- Awareness of vulnerable people in the community (very young, elderly and malnourished)
- Health promotion





Shigella (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

When did people start to get sick with diarrhoea?

How many have fallen sick in the community?

Has anyone died from diarrhoea in this community?

What are the risk habits in the community in regards to water, hygiene, sanitation, disposal of babies, and small children's faeces and food preparation

Who and where are the vulnerable people?

Are there any health services and where?

Is there a health education programme in

place? Who is doing it?

Where do people get their drinking water from? How is collected? How is it stored? Is this area different from where people wash or go to the toilet?

What are the hygiene practices in your community?

Do people know the signs of bloody diarrhoea?

Can people identify the signs of dehydration?

Do people know how to make oral rehydration solution (ORS) or where they can get it from?





Syphilis

Often sexually-transmitted infections (STIs) are missed because people have no symptoms or minor symptoms that disappear quickly but they can infect other people or cause problems when they become (or want to become) pregnant. The germs that cause STIs are carried in bodily fluids including semen, vaginal fluid and blood.

Transmission

- Unprotected vaginal, anal or oral sex with a person who has the infection
- Can affect both men and women
- Can also be passed on during pregnancy from mother to baby

Symptoms

Initial

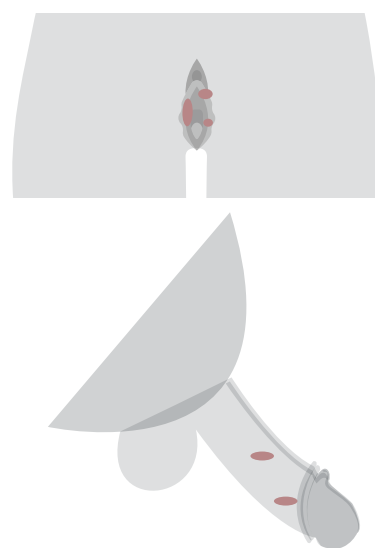
Painless sore or ulcer (chancre) on genitals (penis or outside of vagina) or mouth or anus

Secondary (about 6 weeks later)

- Flu-like illness
- Rash on hands, feet or elsewhere on body
- Small lumps or warts on moist parts of skin (under breast, anus)
- Ulcers in genitals or mouth

Tertiary

- If no treatment syphilis stays in the blood and may come back five to 30 years later
- Serious infection affecting brain, heart or spine
- Death



Prevention

- Sexually-transmitted disease education
- Promotion of safe sex and availability of condoms if culturally appropriate
- Provision of clean childbirth delivery kits

- Handwashing and good hygiene practices
- Health promotion

Vulnerable people

- Adolescents and young adults

In case of a disaster there may be an increase of STIs

- Promotion of safe sex practices and condom distribution
- Promotion of reproductive health education (young people who want to have a baby)
- Promotion of good personal hygiene including handwashing
- Advise anyone who thinks they may have an STI to go to a health facility for help
- If appropriate encourage sexual partners go to clinic for check up (contact tracing)





Syphilis (cont.)

Community-based assessment questions

The risk of transmission of sexually-transmitted diseases is always greater after a disaster due to overcrowding, a high stress environment and when there has been loss of lives. The risk of transmission of sexually-transmitted diseases is always greater after a disaster due to overcrowding, a high stress environment and when there has been loss of lives. When our bodies are overstressed such as during or after a crisis or being in new surroundings, our bodies may be more vulnerable to infection. There can also be a risk of rape in displaced communities.

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community? (do people have access to alcohol or other substances?)

Is this community a result of displaced people?

Who and where are the vulnerable people?

Are there orphans living in this community?

Who is responsible for their care?

Is there overcrowding?

Are there any health services available?

Are there any sexual and reproductive health programmes run in the community?





TB

TB is one of the top 10 causes of deaths in the world. It is caused by bacteria that affect the lungs and can be treated. TB may not be the main priority after an emergency but it is important to restart prevention and treatment programmes as soon as possible to prevent the disease from spreading (especially with displaced people living in overcrowded situations).

Transmission

Germs spread when in close contact with someone with active TB when they cough, sneeze or spit. They can even be spread through talking when tiny droplets of TB bacteria are sprayed in the air (airborne)

Symptoms

Prolonged productive cough for a long time (more than three weeks)
Weight loss
Haemoptysis (blood in spit)
Short of breath

Fevers
Night sweats
Tiredness or weakness
Chest pain
Loss of appetite

* Not all people who are infected will get the disease (inactive or latent TB)
** Brief contact with someone is usually ok, the risk is from prolonged contact

Prevention

Early diagnosis and treatment of people with symptoms
Reduce overcrowding
Use of good coughing and sneezing etiquette (wearing a mask if person known to have the disease)
Isolation of patients with TB in hospital

BCG vaccination of newborns
Hand hygiene
Keep contact with an infected person brief
Disinfection of reusable supplies and surrounding environment
Health promotion
Separation from people with HIV or poor immunity

Who is at risk?

Those in poor living conditions and poor ventilation in house
People with HIV are 20—30 times more likely to get TB
People with low immunity

Higher risk in emergencies when displaced people move into overcrowded living situations

In case of an epidemic

Reduction of overcrowding
Promotion of good ventilation inside homes
Use of good coughing, sneezing etiquette
Hand hygiene
Isolation of patients with TB

Detection and referral of new and suspected cases to health facilities
Awareness of vulnerable people in the community
Health promotion





TB (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

How many people live in the affected community?

What are the risk habits in the community (e.g. prolonged or close contact with people who have:

- TB
- people living with HIV (sexual contact or bodily fluids)
- chronic medical conditions especially respiratory illness
- malnutrition

Are there accessible TB programmes? Are they currently active?

Are there TB health protocols in place? i.e. where do people go for (directly observed treatment) therapy?

Who and where are the vulnerable people?

Are there any health services and where?

Is there a health education programme in place for TB? Who is doing it?





Typhoid

Typhoid fever is caused by bacteria. It can be life-threatening. It enters the body via the gut most commonly due to ingestion of contaminated food or water or direct contact person to person via dirty hands.

People can become carriers once symptoms disappear, spreading via faeces.

Transmission

Faecal oral route - drinking contaminated water or eating contaminated food especially seafood or raw foods

Person to person contact – via dirty hands contaminated by faecal matter

Symptoms

Fever

Tiredness

Headache

Abdominal pain

Diarrhoea in young children or constipation in adults

Decreased appetite

Can develop a rash

Prevention

Promotion of safe clean drinking water including storage

Promotion of basic hygiene practices including handwashing

Promotion of safe sanitation practices

Promotion of good food hygiene including preparation and storage

Social mobilization and behaviour change

Routine vaccination against typhoid

Exclusive breastfeeding up to and beyond six months of age

Vulnerable people

Malnourished children

People with low immunity

People living in overcrowded conditions with poor access to safe water, sanitation and basic hygiene

In case of an epidemic

Start community-based disease surveillance (link to MoH or Early Warning, Alert and Response System (EWARS) as soon as possible)

Increase social mobilization and health promotion activities

Refer anyone who has symptoms to health care facilities

Promotion of safe clean water, collection and storage

Promotion of safe sanitation practices

Promotion of good food hygiene practices

Hand hygiene





Typhoid (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

Has anyone fallen sick with typhoid fever symptoms? How many and where? Have any children fallen sick?

What sanitation facilities (including communal latrines) are available? Do people use them?

What handwashing facilities are available (at households, in markets, etc.)? Do they have soap?

Where are the local health facilities and services? (Include traditional and community carers.)

What are the community's habits, practices and beliefs about hygiene, food preparation and storage, sanitation and water?

Are there health promotion activities already in place?





Yellow fever

Yellow fever is a potentially fatal haemorrhagic disease transmitted by the bite of an infected mosquito. The virus is endemic in parts of Africa and Central and South America. There is a vaccine available.

Transmission

Mosquito bite from an infected mosquito during the day

Symptoms

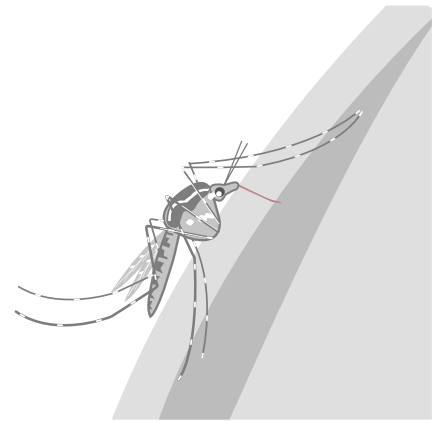
The majority of people infected with yellow fever virus have no illness or only mild illness

Starts with sudden fever, headache and backache, muscle pain, nausea, vomiting and fatigue

Some people develop a second stage of the virus after initial improvement with fever, jaundice (yellow skin), dark urine, abdominal pain, and vomiting

Can also bleed from the gums, nose, eyes or have blood in vomit or faeces

Fatal for between two and five out of 10 people with serious yellow fever



Prevention

Sleeping under mosquito nets

Vaccination of all persons (first dose after nine months of age and then every 10 years) in areas where there is a risk of yellow fever

Prevention of mosquito bites by vector control and use of insect screens on windows

Health promotion

Management of mosquito breeding sites (ponds, water surfaces)

Rapid referral of sick people to a hospital that can provide supportive care

Vulnerable people

Anyone who is not vaccinated against yellow fever is at risk

In case of an outbreak

Get to know health workers who are doing vaccinations and the centres where they are done. Know individuals and families who are hard to reach and help them reach vaccination centres. Help health workers make sure that all individuals at risk are vaccinated (at least 80 per cent of the population should be vaccinated).





Yellow fever (cont.)

Community-based assessment questions

Make a map of the community and mark the information you gather on the map.

- Who and where are the vulnerable people?
- Are there unvaccinated people in the community?
- Are there vaccination campaigns planned?
- Is there a health education programme in place?





Zika

Zika is an emerging mosquito-borne disease. People develop Zika after being bitten by an infected *Aedes* mosquito. It is a daytime biting mosquito.

Transmission

Mosquito bite, mainly during the early morning and late afternoon in sub tropical and tropical areas. This is the same mosquito responsible for dengue and chikungunya.

Zika can be passed from a pregnant woman to the foetus during intercourse (sex) or blood transfusion

Symptoms

Usually none or mild

Fever

Rash

Muscle and joint pain

Conjunctivitis

Headache

General feeling of feeling

unwell and tiredness



Pregnant women

The virus can be transmitted from mother to foetus during pregnancy. This can result in a condition called microcephaly (small head and under-developed brain)

Prevention

Sleeping under mosquito nets (also during the day)

Protective clothing

Promotion of safe storage of water i.e. cover water with lids

Eliminate stagnant water sites, clean up rubbish, ensure drainage around the community

Insect repellent and/or mosquito coils

Safe sex and distribution of condoms or abstinence for six months in an area where Zika virus is present

Vulnerable people

Pregnant women

Women of child-bearing age

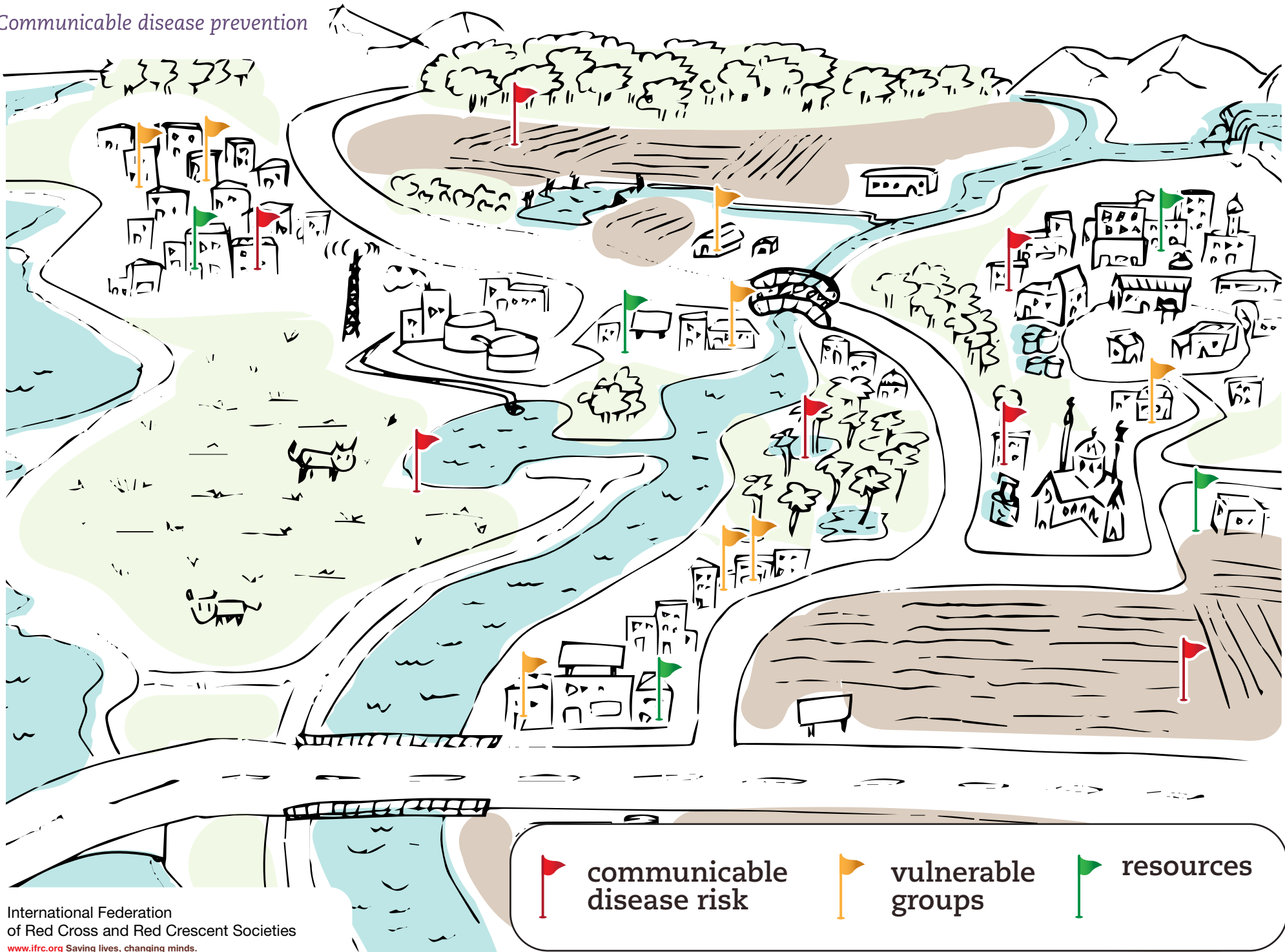
Why do we worry?

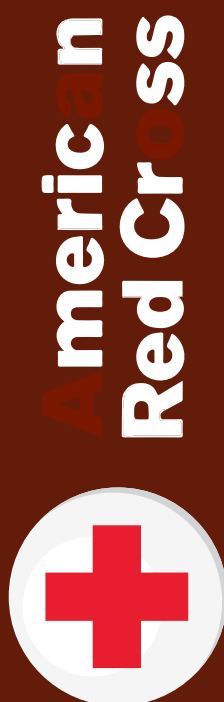
Risk factors can be increased as a consequence of a disaster leading to overcrowded populations and living conditions especially where there are mosquito breeding sites (stagnant water), poor drainage, piles of rubbish.





Communicable disease map





Not enough people donate blood.

Help fill the #MissingTypes.





The first
wealth
is
health.

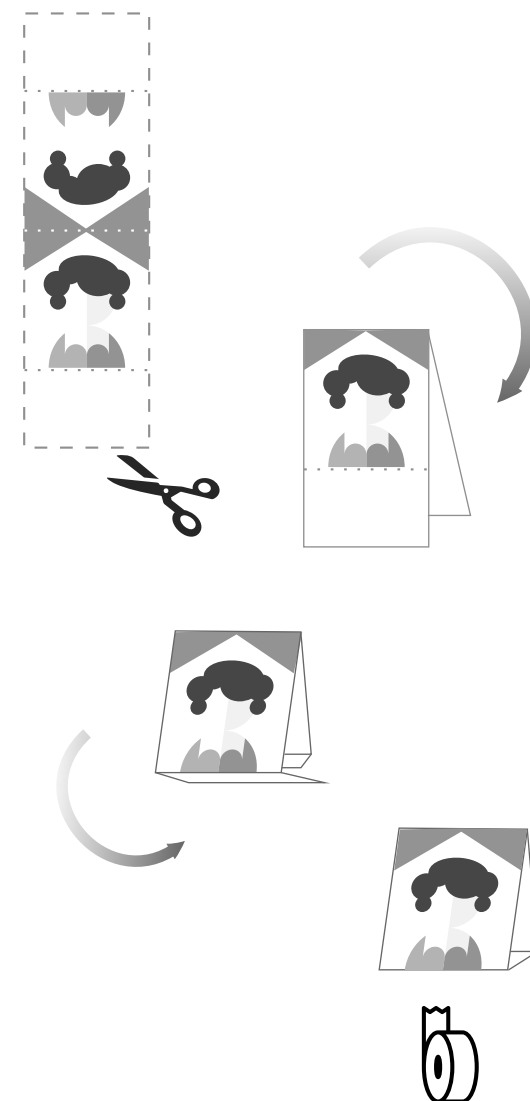
(Ralph Waldo Emerson)





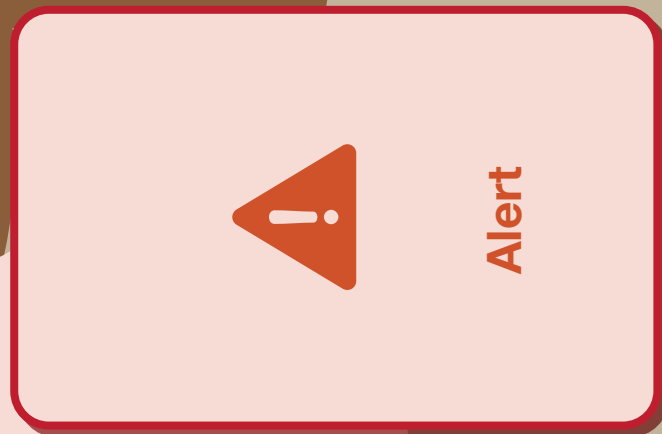
Player pawns

Cut the pawns out along the heavy dotted line. You should have six pawns in total, which you can use to indicate where you are on the game board. Fold the pawns along the light dotted lines and use a piece of sticky tape to stick the bottom pieces of each pawn and to form a triangle-shaped pawn.



Alerting the authorities

GAME



Health
clinic





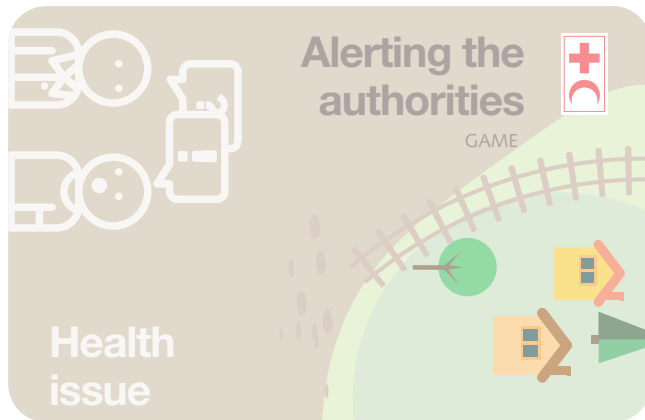
Unsure

No alert

No alert



International Federation
of Red Cross and Red Crescent Societies
www.ifrc.org Saving lives, changing minds.



**Your
community**



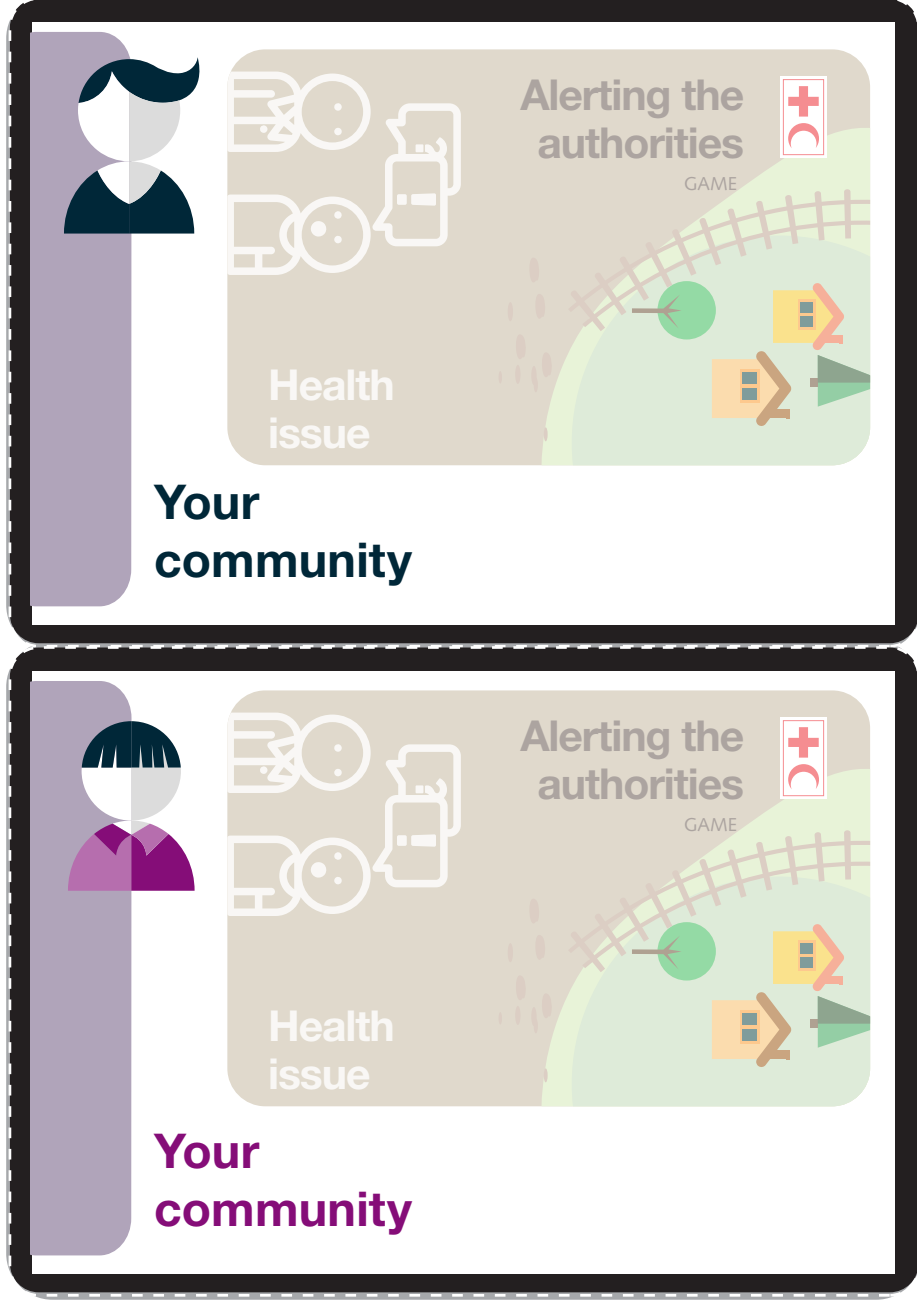
**Your
community**



**Your
community**



**Your
community**





Alerting the authorities game

Goal of the game

To draw, discuss and place all “Health Issue” cards from your deck (on your player pad) onto one of the three spaces (Alert, No alert, Unsure) on the game board.

Setup

Each player chooses a player pawn (B) and matching player pad (C) and places it near one of the five communities on the game board (A). Players can select the same community.

Deal an even amount of “Health issue” cards (D) to each player (dividing up the total amount by the number of players, rounding down). Each player puts the deck of their “Health issue” cards on their player pad (C). To personalize the experience, players may write a community name on their player pad.

Each round...

All players draw the top card of their own “Health issue” deck and discuss the situation on their card in the group. The discussion is whether to put the card in one of the following:

- The “Alert” space next to the Health clinic if the situation is deemed important enough to report to the health clinic or their supervisor.
- The player’s community “No alert” space if it is considered a normal situation that does not need to be reported. This situation may be something you discuss with your supervisor next time they visit.
- The “Unsure” space if players cannot decide upon the importance of the situation on the card.

This principle is similar to three pile sorting (for people familiar with PHAST). The round ends after ALL players have placed their current “Health issue” card on one of the three spaces described above.

Game end

Game ends when all players have drawn and placed their last “Health issue” card.

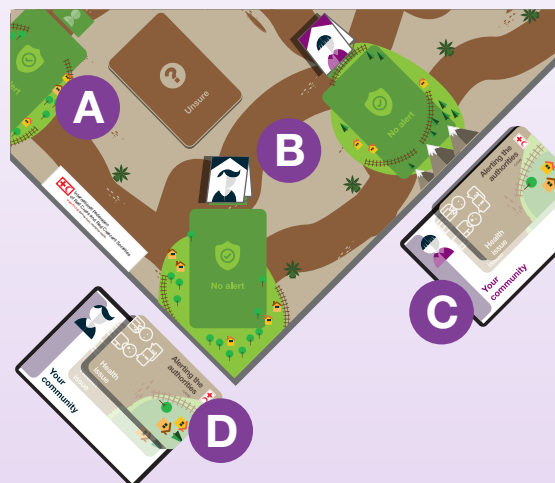
Players are now encouraged to discuss which “Alert” and “No alert” cards deserved to be in their spaces and why. The facilitator should have the knowledge to know whether an alert or no alert is required for each situation.

At the end of the game there should be no cards left in the “Unsure” pile as the discussion should lead to any unsure situations being resolved as either an alert or no alert situation.

Contents

- A) game board (A3 size)
- B) player pawns (max. 6)*
- C) player pads (max. 6)*
- D) 80 “Health Issue” cards

*some assembly required

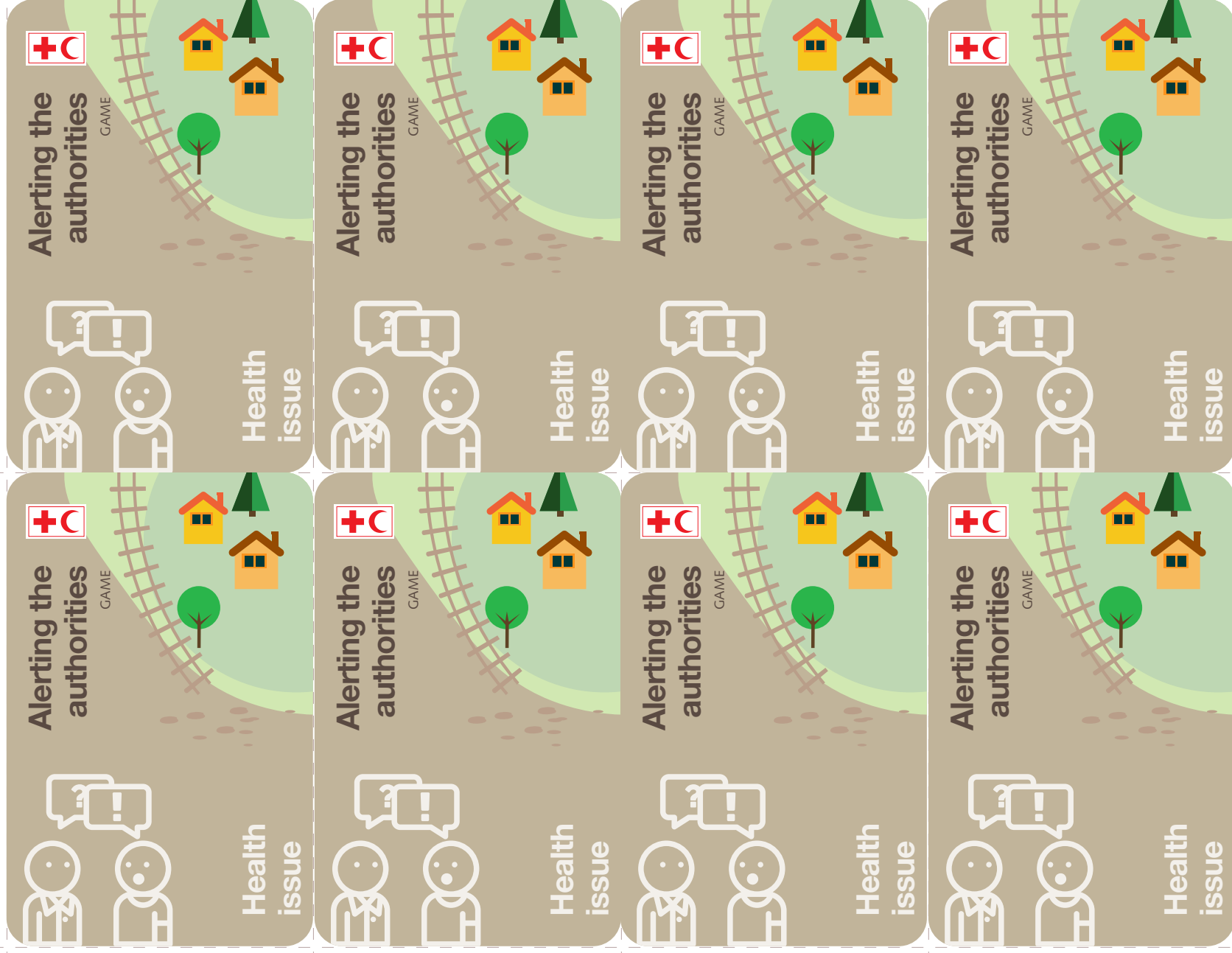


These card backs are optional and can be printed and glued to the front sides of the cards.

Cards (back)



Communicable disease prevention





 Health issue  There is an abrupt onset of fever within several families who live near a pond. Several have complained about joint pain and nausea and show a rash on their skin.	 Health issue  During the recent monsoon rains a lot of people stayed indoors and now some of the women are reporting unusual vaginal discharges and abdominal pains. A few men have complained about burning sensations when peeing.	 Health issue  You have noticed that quite a few adults in your community have yellow eyes and appear tired or clutching their stomachs. These symptoms started appearing after the flush toilets broke down a few weeks ago.	 Health issue  You have spotted several needles in the local trash pit just outside of town. Rumours about a group of drug-using men have been going around as well as talk of several men vomiting and complaining about aching joints.
 Health issue  A young couple has shared concerns about small blisters on their genitals. When questioned further, they admitted to have had unprotected sex with others throughout the community.	 Health issue  A number of people in your community have been coughing and sneezing lately. There is a festival planned in four days to which many people normally go.	 Health issue  After a local wedding, a good number of people have complained about diarrhoea, fever and headaches. The next week a drought is expected and water will be scarce.	 Health issue  After a string of rainy days last month, some local women (both pregnant or sexually active) have come down with a fever, rash and muscle pains.





Communicable disease prevention

Health issue

A young farmer who works long hours has come to you complaining about his urine being darker than normal.

Health issue

You have a lecture on handwashing to which the whole community is invited. Afterwards, several people come to you, concerned that they normally do not wash their hands.

Health issue

You see two children petting a cat on the street. One of them scratches her head and wipes her runny nose with her hand.

Health issue

A young man reports to you his concerns of catching HIV from sitting on the public flush toilet.

Health issue

A strong man comes to you with his wife. She says he is suffering from stomach aches but thinks it is because he eats a lot of spicy foods.

Health issue

One of the workers has fallen off a ladder onto his chest and is complaining about having breathing difficulties.

Health issue

You spot a rat running through the market. This is the second one you have seen in a month.

Health issue

A woman is concerned for her young children contracting Hepatitis E after her husband (who is a cook) came home with a loss of appetite and dark rings under his eyes. He has been working a lot lately.





Communicable disease prevention

Health issue

A vendor at the market has complained about an awful smell coming from the VIP toilets. You investigate and find the facility has broken down and is overflowing with waste.

Health issue

The community elder has come to you complaining about headaches and aching joints.

Health issue

There was a party last night which half the village attended. Later that night two boys were vomiting.

Health issue

After yesterday's rain storm, you walk through the community to check up on a family whose youngest child is in bed with a fever.

Health issue

Someone comes into your office and sneezes without covering their mouth.

Health issue

Two teenagers were playing football this morning when one ran into traffic and was hit by a car. You are heading over to administer first aid.

Health issue

You find a needle lying on the floor of the local clinic.

Health issue

A young couple comes to you to ask if they should start wearing condoms if they have sex.





 Health issue  Heavy rains are expected over the next few days and there is a pregnant woman about to give birth around the same time.	 Health issue  One of the elder women in your town passed away in her sleep last night and the community wants to hold a ceremony for her today.	 Health issue  Two teenagers admit to having unprotected sex a while ago and now the girl is pregnant. She comes to you for advice.	 Health issue  Three people are sick in bed this morning. After asking around, all three had been seen drinking heavily at a party last night.
 Health issue  A mother comes to you with her young son who is covered in spots. She is afraid he is dying. The child seems otherwise fine.	 Health issue  Two schoolgirls tell you on the street that there is a small lake behind their school they avoid on their way home because there are many mosquitos present.	 Health issue  The local football player comes to you with a headache and looking pale. He's afraid he'll miss the match for which he has been practising. You remember him from last year when you vaccinated him against yellow fever.	 Health issue  A woman comes to you complaining about having diarrhoea. When asked how long she's had it, she says since this morning.





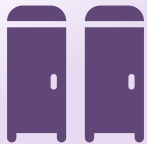
Communicable disease prevention

 Health issue  You spot two children sharing a piece of fruit. They are handing it back and forth with dirty hands.	 Health issue  A woman whispers to you embarrassed that she scratched her crotch a few times today. She is concerned she caught a sexually-transmitted disease. She is 60 years old and a widow.	 Health issue  You administer first aid to a young boy who was chasing a dog when he fell down and scratched his knees.	 Health issue  Checking up with several injured people after a recent but minor earthquake you find that they are doing well and on the way to recovery.
 Health issue  The local health supervisor calls you after a local community nearby has reported an outbreak of the measles. You have noticed nothing out of the ordinary in your community.	 Health issue  An exceptionally wet and cold season is predicted and you check your inventory on medicine and supplies. All is well.	 Health issue  A friend of someone with HIV comes to you concerned that they have HIV now as well as they hugged and shook hands.	 Health issue  An especially hot day today. It hasn't rained in a while but there are no reports about water shortages or loss of crops.





**Health
issue**



You hear from the community leader that a 35 year old man died from diarrhoea yesterday.



**Health
issue**



Since the floods caused us to move into a temporary camp many children are getting acute respiratory infections.



**Health
issue**



Since the earthquake rubbish is piling up behind the tents and children are playing amongst it. There are more rats than usual and an old man was bitten on the hand and now has a fever.



**Health
issue**



The camp is getting bigger every day and the latrines are overflowing. Everyone is now going to the toilet in the bush and water and we have many cases of diarrhoea.



**Health
issue**



There is a five year old child in the community who cannot walk and seems very weak. He is having trouble swallowing.



**Health
issue**



A 70 year old woman passed away this morning. She has been frail for the last several months now.



**Health
issue**



Since the floods there has been a shortage of clean water. Bottles of fresh drinking water have been handed out, but people are not washing themselves adequately.



**Health
issue**



More refugees are entering the camp, some with wounds from the nearby conflict. Others are coughing and appear feverish.



Print double-sided (if possible) and cut out these cards along the dotted lines. Alternatively these can be printed separately and glued to the back sides of the cards.

Cards



Communicable disease prevention

Health issue

Villagers are afraid that the nearby volcano will erupt as this was predicted by village elders several generations ago. There is no unusual activity otherwise.

Health issue

Some of the water sources have been producing dark brown water while others have stopped running water altogether.

Health issue

One of the cows mysteriously died last night. She was a young cow and there are no signs of sickness or wounds.

Health issue

Some of the fishermen have reported a drop in fish populations. They are worried about a tsunami (which occurred three years ago).

Health issue

An armed conflict has erupted in the nearby country, raising concerns among the community that it might spill over to this country. There is no official word from the government.

Health issue

One of the houses in the community collapsed last night, raising fears of an earthquake. No other structures have been damaged.

Health issue

Recent floods have retreated leaving the community to discover the body of a young man who went missing during the period.

Health issue

A family reports that bugs have been found in their food supply.



Print double-sided (if possible) and cut out these cards along the dotted lines. Alternatively these can be printed separately and glued to the back sides of the cards.

Cards



Communicable disease prevention

Health issue

There is an unusual amount of garbage piling up in the local trash pits since the men responsible have been called into military service.

Health issue

A baby was born in the community last night and there are people talking that it has a very small head and looks different to the other babies.

Health issue

There is a blood donation event next week, but many residents are afraid they might catch HIV.

Health issue

After the recent mudslides water sources were polluted and bottles of drinking water were handed out. That was four months ago and even though water sources are clear again, the community is worried.

Health issue

Twin babies died last night, causing fears within the community about a potential disease targeting the vulnerable.

Health issue

Medicine is running low in the community.

Health issue

200 kilometers from your community another community has been struck by floods. You have family living there and are deciding to go and help.

Health issue

After the recent shigella outbreak the community had taken steps to attend to healthier hygiene, but now six months later people are back to not washing their hands.





 Health issue  You have been asked to present your community health status next week in the capital but after a recent fire all of your records are lost.	 Health issue  The oldest living person in your village has asked if you can help her get to the capital.	 Health issue  An accident at the local construction site has left two men with broken bones.	 Health issue  A group of young men and women have come to you with a request for vaccinations. They are nervous after the outbreak of the flu in a nearby community.
 Health issue  An old man comes to you complaining about a stomachache. He is afraid he might have shigella but says his stool is normal.	 Health issue  A young woman is afraid that her neighbours might have a respiratory disease as they have been seen wearing masks.	 Health issue  There have been refugees begging in the streets of your community raising questions from people concerned that they might carry infectious diseases.	 Health issue  A concerned mother comes to you saying that her son helped another boy up after he had fallen down at school. The boy had head lice and now she's worried her son might have too.





 Health issue A few people from your community want to know if they have to wash their hands after touching someone with Hepatitis A.	 Health issue A local farmer is concerned about touching the dirt with his hands as he has learnt that salmonella can be contracted from it.	 Health issue A bus driver who ended up in an accident was bandaged by someone with HIV and is now worried he might be a carrier.	 Health issue A friend of a young woman, known to have herpes, is concerned that she has herpes now as well. She has recently kissed her mother, boyfriend and one more boy.
 Health issue A mother who saw her son scratch his head scratched her head later on and is worried she has head lice.	 Health issue A fisherman killed several mosquitoes in the last week and wants to know if he contracted chikungunya or malaria. He has no symptoms.	 Health issue Hot and dry weather has been forecast for the next month and water levels are low in the river and water tanks.	 Health issue A pregnant woman is afraid that her child might have been infected with Zika. She shows no symptoms.





Effective message scorecard

Scorecard to assess the quality of messages. For each row, assign a score of 1, 2 or 3. Add the score from each row for a total score.

CRITERIA	3	2	1	Total
Simplicity: Use of images. Clear design. Easy wording				
Communication of a benefit: Can read or see the good gained by doing the healthy behaviour				
Call to action: Clear advice on what to do to achieve the benefit. (Inspiring, motivating)				
Context appropriate: Appropriateness for target community, based on evidence of why the behaviour is not already happening, and on cultural norms of community. In local language showing appropriate imagery, informed and tested by community.				
Scoring: Add the scores for each criterion to achieve a total score. Out of a total of 12 points, an acceptable message should not score below 9.				





How to do doer/non-doer analysis

Doer/non-doer analysis is an assessment tool to be used when you see that there is a specific behavioural barrier in a community. We use doer/non-doer analysis to better understand what beliefs or misbeliefs are causing or allowing a bad or unhealthy behaviour to continue.

To identify WHY people continue to engage in a specific unhealthy behaviour or refuse to practise a specific healthy behaviour, we use doer/non-doer analysis to learn from the doers (those doing the healthy behaviour) to better identify norms, beliefs and strategies to influence the non-doers (those not doing the healthy behaviour) to adopt the healthy behaviour.

Do you have a specific behaviour in mind for your community to explore? If yes, let us get started.

- 1 We first need to identify who in the community already practises the specific healthy behaviour and who does not and interview each of these people individually to examine why this is so.

A doer/non-doer analysis consists of a series of seven questions we ask individuals about the reasons behind why they practise or do not practise a specific healthy behaviour. There are many possible reasons and when we uncover one or more of these reasons, we get a better idea of what we as eCBHFA volunteers need to do to help communities actually start practising healthy behaviours.

One of the key advantages of the doer/non-doer behaviour analysis is that it can be conducted using the focus group discussion approach and/or the household interview tool. These methods allow access to community members and provide a trusted purpose for asking questions about behaviour, allowing more useful information to be gathered.

Materials – participants – time when conducting doer/non-doer analysis



- The doer/non-doer analysis questions embedded in the interview guideline questions and/or the household interview questions
- The doer/non-doer Excel workbook
- Tool
- Pens
- Clipboard, paper



- A group of community members who all either DO practise a key healthy behaviour or all DO NOT practise a key healthy behaviour
- Two interview teams, each team made up of:
 - one eCBHFA volunteer interviewer
 - one eCBHFA facilitator
 - two eCBHFA volunteer note-takers



15 - 30 min.

initial focus group discussion (FGD) to identify doers and non-doers

30 - 40 min.

for EACH individual interview of all available doers and non-doers

Reflection:

- 1 Look at the doer/non-doer analysis questions. How easy are they to embed in the focus group discussion questions or household interview questions?
- 2 How can you adapt them to the context while still getting the essential behaviour change information we need?





How to collect the information

If you are already conducting a FGD or a household interview, you should include these questions as part of your interview or discussions with the community. However, you will need to ensure that all the members of a specific focus group discussion are either identified as doers (those who practise the healthy behaviour or do not practise unhealthy behaviours) or non-doers (those who do not practise the healthy behaviour or refuse to stop an unhealthy behaviour).

Doer/non-doer analysis within FGD

If you are not conducting a FGD or household interview, you can and should still explore and analyse who in the community is doing a certain behaviour, who is not and which issues might be preventing healthy behaviours taking place.

To conduct the doer/non-doer analysis, identify the target group where you want to focus your attention. Pre-identify participants for a behaviour FGD by first assessing each person one-on-one using the questions in Part A of the doer/non-doer analysis tool.

When you have at least three to four participants all identified as a doer or non-doer, conduct your FGD using the questions in Part B of the doer/non-doer analysis tool.

Analyse your results by looking for information about the people, organizations, policies, cultural and religious beliefs and rules that affect the community and their behaviours or belief in accomplishing a behaviour. Review the results of the doer answers against the answers of the non-doer answers to see the differences. The results can inform what an eCBHFA volunteer can do and say to help address barriers to healthy behaviour practice.

Doer/non-doer analysis within household interviews

If you are conducting the doer/non-doer analysis as part of a household interview, complete one sheet for each household member over the course of one or several visits. The responses may take more time as some people in the household may not feel comfortable discussing barriers to change at home among family members. Ensure household members that eCBHFA volunteers keep private information private and set up a time when you might be able to help them identify barriers to healthy behaviour.

Complete one sheet for each household member asking each household member questions in Part A and then immediately ask the questions in Part B. Take notes and listen without comment or judgement of any kind. Record your answers on the doer/non-doer analysis sheet for wider review of all community members for a community behaviour change solution.

Reflection:

- 1 Why is it important to have a group of people identified all as doers or all non-doers for a behaviour change FGD?
- 2 Does the note-taker record responses for each individual participant for a doer/non-doer analysis FGD?
- 3 Does the volunteer record responses for each household member on individual sheets for a doer/non-doer analysis during a household interview?





How to collect the information (cont.)

Review the notes taken after the FGD or household interview

Immediately after each FGD or each household interview, the interviewer and the note-taker(s) should thoroughly review the responses given by the interviewees/participants and what the note-takers(s) wrote down, as well as the facilitator's recollections of key discussion points. They should confirm that they have recorded the important points, and fill in any other essential information that may have been missed.

This is a vital exercise and must be carried out immediately after each FGD and household interview: details can easily be forgotten if the notes review process is delayed, or if the team reviews all of the FGD or household interview notes at the end of the day.

Once again, after reviewing and making appropriate changes to the notes, the facilitator and note-taker(s) should then write down their key impressions of what they learnt during each FGD or household interview.

Fill in the "Information compilation worksheet"

Using the information worksheet for each completed activity (focus group discussion and household interviews), transfer the information to the corresponding boxes in the information compilation worksheet (see the Assessment module).

Additional "focus group discussion" or "household interview" rows may need to be added to the information compilation worksheet, depending on the number of each that were conducted.





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